

INTRODUCTION PATIENT CASE HISTORY

Today's Date: 1/14/21

PATIENT INFORMATION

Name: (First MI Last) Jeff L. Plummer Preferred Name: Jeff
Address: [REDACTED] City: Lotz State: FL Zip: 33548
Date of Birth: [REDACTED] Gender: ☒ Male ☐ Female Social Security # [REDACTED]
Home: N/A Mobile: [REDACTED] Work: N/A
Email: [REDACTED]
Preferred Method of Contact: ☒ Text ☐ Email ☐ Phone - Home, Mobile, or Work ☐ Other: _____

*Referred By: (Name) Farah & Farah
☐ Family ☐ Friend ☐ Co-Worker ☐ Doctor ☐ Other: Attorney

Race & Ethnicity: (Choose up to 2)

- ☐ African American or Black
☐ American Indian or Alaskan Native
☐ Asian
☐ Hispanic or Latino
☐ Native Hawaiian or Other Pacific Islander
☒ White
☐ Decline

Preferred Language:

- ☒ English
☐ Spanish
☐ Other: _____
☐ Decline

EMERGENCY CONTACT INFORMATION

Name: (First MI Last) Betty Plummer Primary Care Physician: _____
Home: _____ Mobile: _____ Doctor's Phone: _____
Relationship:
☐ Child ☒ Parent ☐ Spouse ☐ Other: _____

FINANCIAL INFORMATION

Is today's visit the result of an accident?

- ☐ No ☒ Auto ☐ Work ☐ Other: _____

Will we be working with insurance? ☐ No ☒ Yes (Details)

Primary: _____ ID#: _____
Secondary: _____ ID#: _____

Where would you like statements sent?

- ☐ Self ☐ Other (Details below)

Name: _____
Address: _____
Phone: _____ Email: _____

It is Usual and Customary to Pay for Services as Rendered Unless Otherwise Arranged

Account No: _____

ACCIDENT/INJURY QUESTIONNAIRE

Name: (Last, First MI) Plummer, Jeff L.

Today's Date: 1-14-21

AUTOMOBILE ACCIDENT - ADDITIONAL INFORMATION

- Was anyone else in the vehicle with you? ☒ No ☐ Yes - (Number of people) _____
- You were? ☒ Front seat - Driver / Passenger ☐ Rear Seat - Behind Driver / Middle / Behind Passenger / 2nd Row / 3rd Row
- Name of Driver, if not self: _____ Name of Driver of other vehicle: _____
- Did airbags deploy? ☒ No ☐ Yes Did Police arrive? ☒ No ☐ Yes Using Seatbelt? ☐ No ☒ Yes
- Did you strike the windshield or object in car? ☐ No ☒ Yes - (Describe) Head rest
- Were you knocked unconscious? ☒ No ☐ Yes (How long?) _____
- Where was your vehicle impacted? Front ☒ Rear ☐ Passenger Side / Driver's Side / Other: _____
- Where was the other vehicle impacted? ☒ Front ☐ Rear ☐ Passenger Side / Driver's Side / Other: Pass Side
- Your Auto Ins: _____ Policy #: _____ Claim #: _____ Phone #: _____
 - Address: _____ City: _____ State: _____ Zip: _____
- Other's Auto Ins: _____ Policy #: _____ Claim #: _____ Phone #: _____
 - Address: _____ City: _____ State: _____ Zip: _____

WORKER'S COMPENSATION INJURY - ADDITIONAL INFORMATION

Employer: _____ Occupation: _____ Claim #: _____
Address: _____ City: _____ State: _____ Zip: _____
Contact Person: _____ Phone: _____ Email: _____

GENERAL ACCIDENT/INJURY INFORMATION - (PLEASE USE THE REVERSE SIDE OF THIS PAGE IF ADDITIONAL SPACE IS NEEDED)

Date of Accident: 1/14/21 Time: 10:15 AM/PM

Please describe the accident in as much detail as possible? I was trying to pull out into traffic and had my head turned as far to the left as possible to watch traffic so I could merge/pull out.

Before the accident/injury:

- Have you ever had any complaints in the involved area before? ☒ No ☐ Yes
 - If yes - Were they present at the time of the accident/injury? ☒ No ☐ Yes
 - If yes - Summarize these complaints prior to the accident: _____
- Were you capable of performing all of your work activities without restriction? ☐ No ☒ Yes

At the time of the accident/injury:

- Did you feel pain immediately after the accident? ☒ No ☐ Yes ☒ Later that day ☐ Next day ☐ When? _____
- Were you taken anywhere after the accident? ☒ No ☐ Yes ☐ Later that day ☐ Next day ☐ When? _____
 - If yes, How? _____ Where? _____
 - If yes, Did you receive treatment? ☐ No ☐ Yes - (Describe) _____

Since the accident/injury:

- Are your symptoms: ☐ Improving? ☒ Getting Worse? ☐ The Same?
- Are your work activities restricted as a result of this accident/injury? ☐ No ☒ Yes - (How?) can't sleep/neck gets stiff/so on
- Have you missed any work since this accident? ☒ No ☐ Yes - (Dates?) _____
- Have you retained an Attorney? ☐ No ☒ Yes - Name: Farah & Farah Phone: _____
 - Address: _____ City: _____ State: _____ Zip: _____

HISTORY OF PRESENT ILLNESS

HISTORY OF PRESENT ILLNESS (Please describe)

Major Complaint: stiff, sore neck, cracking
popping in neck, can only sleep on
my back. Arms tingle when sleeping on side

Secondary Complaints: _____

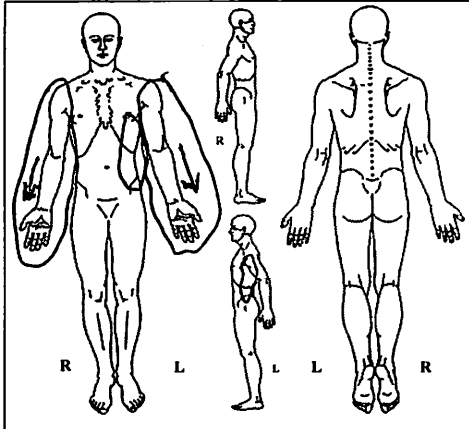
When did it start? 1/5/21

What happened? had problems sleeping

Which daily activities are being affected by this condition? all, depending on situation

MAJOR COMPLAINT

Location of Symptoms and Radiation



P ☒ Pain
N ☒ Numb
S ☐ Spasm

T ☐ Tender
H ☐ Hypoesthesia

Grade Intensity/Severity:

- ☐ None (0/10)
☐ Mild (1-2/10)
☐ Mild-Moderate (2-4/10)
☐ Moderate (4-6/10)
☒ Moderate-Severe (6-8/10)
☐ Severe (8-10/10)

Frequency:

- ☒ Off & On
☐ Constant

Quality:

- ☒ Sharp
☐ Stabbing
☐ Burning
☐ Achy
☐ Dull
☒ Stiff & Sore
☐ Other: _____

Does it radiate?

- ☐ No ☒ Yes (Please indicate on drawing)

Improves with:

- ☐ Ice
☐ Heat
☒ Movement
☒ Stretching
☐ OTC Medications: _____
☐ Other: _____

Worsens with:

- ☐ Sitting
☐ Standing/Walking
☒ Lying Down/Sleeping
☐ Overuse/Lifting
☒ Other: spasms throughout day

Previous Treatment:

- ☐ None
☒ Chiropractor lower back
☐ Medical Doctor _____
☐ Physical Therapy _____
☐ ER/Urgent Care _____
☒ Orthopedic Broken Arms/foot
☐ Other: _____

Previous Diagnostic Testing:

- ☐ None
☒ X-rays Arms/foot/back?
☒ MRI ? (Chiropractor)
☐ CT _____
☐ Other: _____

*Women: Are you pregnant?

- ☐ No Last Menstrual Period: 1/1/
☐ Yes Due date: 1/1/

Present Illness Comments:

Prescription Medications & Supplements: ☒ None

☐ Yes (List - Name, dosage, frequency) _____

Allergies to Medications: ☒ No known drug allergies

☐ Yes (List - Name and reaction) _____

PAST, FAMILY, AND SOCIAL HISTORY

PAST MEDICAL HISTORY

Have you ever had any of the following? (Please select all that apply and use comments to elaborate.)

Illnesses:

- ☐ Asthma
☐ Autoimmune Disorder (Type) _____
☐ Blood Clots
☐ Cancer (Type) _____
☐ CVA/TIA (stroke)
☐ Diabetes
☐ Migraine Headaches
☐ Osteoporosis
☐ Other: _____

Hospitalizations: (Non-surgical with Date)

Rocky Mountain Spotted Fever(?)

Surgeries: (If yes, provide type & surgery date)

- ☐ Cancer _____
☐ Orthopedic
 Shoulder – R / L _____
 Elbow/Forearm – R / L _____
 Wrist/Hand – R / L _____
 Hip – R / L _____
 Knee – R / L _____
 Ankle/Foot – R / L _____
☐ Spinal Surgery
 Neck: _____
 Back: _____
☐ Other: _____

Medical History Comments:

Injuries:

- ☒ Back Injury
☒ Broken Bones
☐ Head Injury
☐ Neck Injury
☐ Falls
☐ Other: _____

FAMILY HISTORY (Please mark X to all that apply and use comments to elaborate.)

- ☒ Unknown ☐ Unremarkable

	Mother	Father	Sibling1	Sibling2	Sibling3	Child1	Child2	Child3
Gender	F	M						
Age at death (if Deceased)								
Aneurysms								
CVA (Stroke)								
Cancer								
Diabetes								
Heart Disease								
Hypertension								
Other Family History								

Family History Comments:

SOCIAL AND OCCUPATIONAL HISTORY

Marital Status: ☒ Single ☐ Married ☐ Divorced ☐ Other

Children: ☐ None ☒ 1 ☐ 2 ☐ 3 ☐ 4 ☐ Other: _____

Student Status: ☐ Full Student ☐ Part Student ☒ Non-Student

Highest level of Education: ☒ High School ☐ College Grad.

☐ Post Grad. ☐ Other: _____

Employed: ☐ No ☒ Yes (Occupation) Self employed

Dominant Hand: ☒ Right ☐ Left ☐ Ambidextrous

Smoking/Tobacco Use: If current smoker, amount = NO

☐ Every Day ☐ Some Days ☐ Former ☐ Never

Alcohol Use:

☐ Every Day ☐ Weekly ☒ Occasionally ☐ Never

Caffeine Use:

☒ Coffee ☐ Tea ☐ Energy Drinks ☐ Soda ☐ Never

Exercise frequency:

☐ Daily ☐ 3-4xs/week ☒ 2-3xs/week ☐ Rarely ☐ Never

Social History Comments: _____

Today's Date: _____ Patient Name: _____ Account No: _____

REVIEW OF SYSTEMS

Are you *currently* experiencing any of these symptoms? *(Please select all that apply and use comments to elaborate.)*

☐ Fever
☒ Fatigue
☐ Other: _____
☐ None in this Category

☐ Difficulty Breathing
☐ Cough
☐ Other: _____
☒ *None in this Category*

[illegible]

☒ Joint Pain/Stiffness/Swelling
☒ Muscle Pain/Stiffness/Spasms
☐ Broken Bones _____
☐ Other: _____
☐ None in this Category

☐ Eye Pain
☐ Blurred or Double Vision
☐ Sensitivity to Light
☐ Other: _____
☒ *None in this Category*

☐ Dizziness or Lightheaded
☐ Convulsions or Seizures
☐ Tremors
☐ Other: _____
☒ *None in this Category*

☐ Frequent or Recurrent Headaches
☐ Ear - Ache/Ringing/Drainage
☐ Hearing Loss
☐ Sensitivity to Loud Noises
☐ Sinus Problems
☐ Sore Throat
☐ Other: _____
☒ None in this Category

☐ Nervousness/Anxiety
☐ Depression
☒ Sleep Problems
☐ Memory Loss or Confusion
☐ Other: _____
☐ *None in this Category*

☐ Infertility
☐ Recent Weight Change
☐ Eating Disorder
☐ Other: _____
☒ None in this Category

☐ Frequent or Painful Urination
☐ Blood in Urine
☐ Incontinence or Bed Wetting
☐ Painful or Irregular Periods
☐ Other: _____
☒ *None in this Category*

☐ Excessive Thirst or Urination
☐ Cold Extremities
☐ Swollen Glands
☐ Other: _____
☒ None in this Category

- ☐ Loss of Appetite
- ☐ Blood in Stool or Black Stool
- ☐ Nausea or Vomiting
- ☐ Abdominal Pain
- ☒ Frequent Diarrhea (*more than normal*)
- ☐ Constipation
- ☐ Other: _____
- ☐ None in this Category

☐ Rash or Itching
☐ Change in Skin, Hair, or Nails
☐ Non-healing Sores or Lesions
☐ Change of Appearance of a Mole
☐ Breast Pain, Lump, or Discharge
☐ Other: _____
☒ *None in this Category*

☐ Chest Pains/Tightness
☐ Rapid or Heartbeat Changes
☐ Swelling of Hands, Ankles, or Feet
☐ Other: _____
☒ *None in this Category*

☐ Food Allergies
☐ Environmental Allergies
☐ Other: _____
☒ *None in this Category*

Patient or Guardian Signature

Date _____

1-14-2

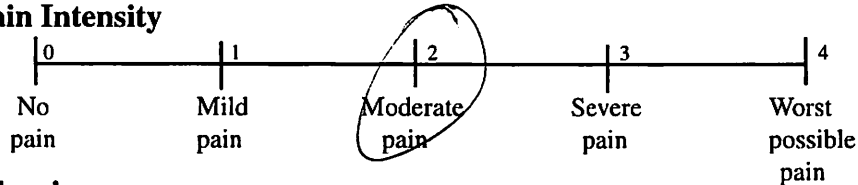
Functional Rating Index

For use with Neck and/or Back Problems only.

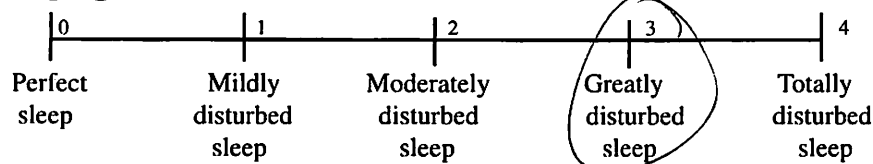
In order to properly assess your condition, we must understand how much your neck and/or back problems have affected your ability to manage everyday activities.

For each item below, please circle the number which most closely describes your condition right now.

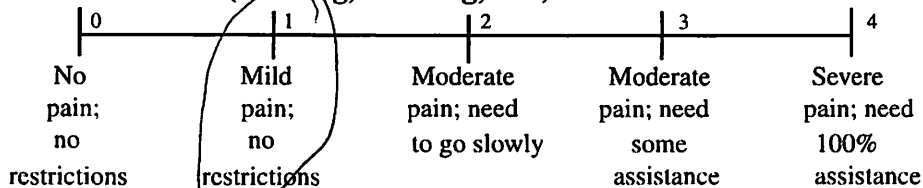
1. Pain Intensity



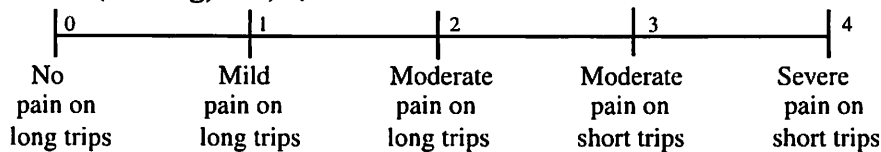
2. Sleeping



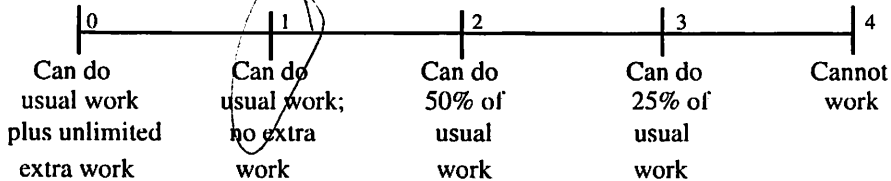
3. Personal Care (washing, dressing, etc.)



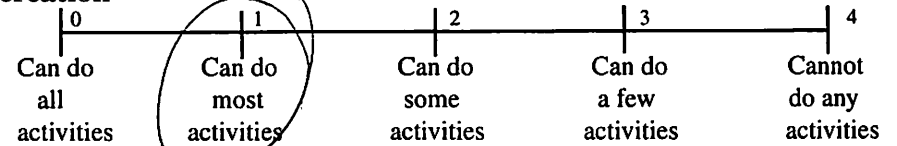
4. Travel (driving, etc.) *haven't taken one*



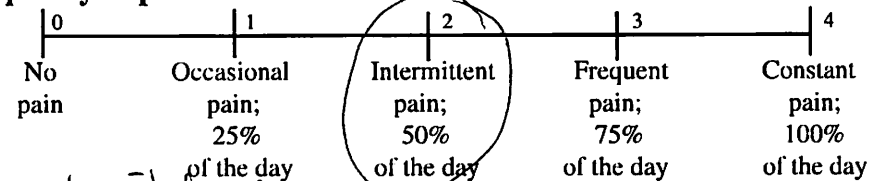
5. Work



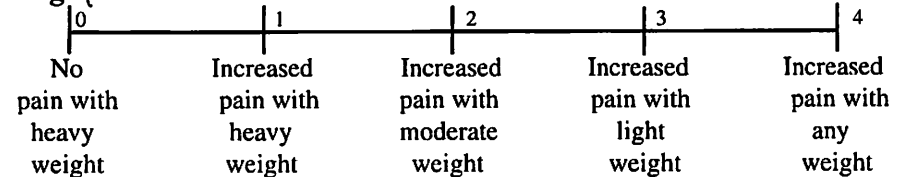
6. Recreation



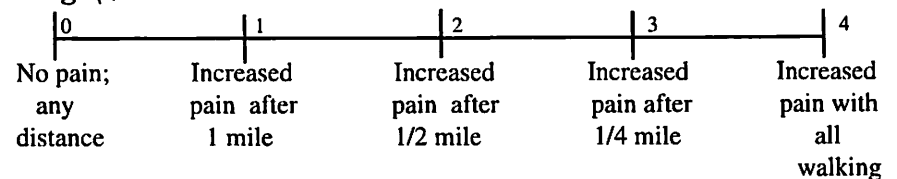
7. Frequency of pain



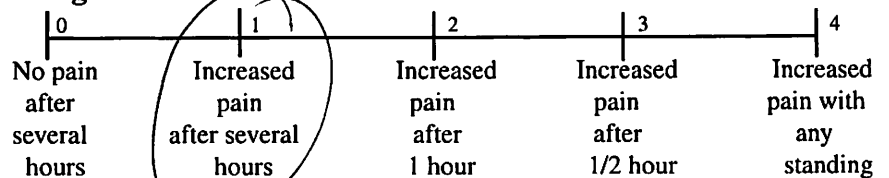
8. Lifting *haven't done*



9. Walking *haven't walk a mile*



10. Standing



Name Jeff L. Plummer

PRINTED

ID#/SS#

Plan ID

Total Score

Jeff L. Plummer
Signature

1-14-21
Date

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PHYSICAL EXAMINATION

Patient: Jeffery Plummer

Examiner: Dr Heidi Pickering DC

Patient No: 1042

Date: 1/14/2021

Attorney

Vitals	L	R	X-Ray Finding
B/P	131/66		Misalignment in area of complaint
Pulse	76		Findings correlate w complaint level
Lungs			DJD present
Weight	83	98	Curvature
Height	72" 6"		Other:
Appearance	Distress		
Well Groomed	No Apparent		
Well Nourished	Mild		
Obese	Moderate		
Morbid	Severe		
* Special Attention			
Diagnostic Workup			
History Notes	19+ Tension 1.4.21		
Onset	Days _____ Weeks _____ Mos _____ Years _____		
Chief Complaint	neck 3/4 H A radiation (L) (R) to hand		
Secondary Complaints	URS (L)		
Plan of Care	Resolution: Days: 120 90 60 45 30 # 30 24 20 18 12 Other Re-evaluation: 30 days Other Frequency: 4x 3x 2x 1x 1/2x 1/3x 4wk 5wk 6wk Other Procedures: ADJ IST ET EXT ADJ Other Patient Instructions: BRICE Home Exercises Dos & Don'ts DME Restrictions Other Referrals: None Other Future Tests: None Other Contraindications: N/Y Complicating Factors: N/Y		
Goals:	Short Term: Pain Function Long Term: Pain Function		
Cervical Spine	Left Adson's Bakody's + Compression + Distraction Dejerine's: + - Soto Hall: + - *Slump Test: + - Right		
Dorso-Lumbar Spine	Left + Kemp's + Lasague's/SLR Braggard's FABER Lewin Ely's Deerfield *Slump Test: + - Right		
X-Rays Ordered	Cervical: 2 views 3 views 5 views Flex Ext Thoracic: 2 views 3 views Lumbar: 2 views 3 views 5 views Flex Ext Other		
Posture	Unremarkable Cervical Lordosis: N ↑ ↓ Thoracic Kyphosis: N ↑ ↓ Lumbar Lordosis: N ↑ ↓ Antalgic: Scoliosis:		
Gait & Station	Normal Abnormal Romberg Test: + - Heel Walk Toe Walk Trendelenburg		
Muscle Strength (0-5)	Left *Hand Grasp Deltoid Biceps Wrist Ext. Wrist Flex Finger Ext Finger Abd Finger Add Right		
Muscle Measurements	Left Biceps Forearm Thigh Calf Right		
Mood & Affect	Normal Depressed Anxious Agitated		
Orientation	A&O x 3 Disoriented		
Coordination	Balance: WNL Abn Tandem/Walk: WNL Abn Finger/Nose: Left + - Right + -		
Dermatomes	Left C4 C5 C6 C7 C8 T1 Right L1 L2 L3 L4 L5 S1		
Cranial Nerves	Left I II III IV V VI Right VII VIII IX X XI XII		
Cardiovascular			
Lymphatic			
Skin			
Reflexes (0-4)	Left Babinski Hoffman Biceps Brachioradialis Triceps Patellar Achilles Right		
Extremities	UPPER L R PAIN L R SPASM L R Motion L R Shoulder Elbow Wrist Hand LOWER L R PAIN L R SPASM L R Motion L R Hip Knee Ankle Foot		

Diagnosis Codes - ICD10

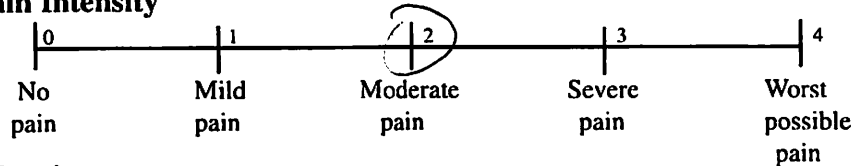
CERVICAL REGION	THORACIC REGION	LUMBAR REGION	PELVIC/SACRAL REGIONS
M99.01 Segmental Dysfunction M1 M54.2 Cervicalgia M2 M62.838 Other Muscle Spasm M2 R51 Headache M2 <u>Spondylosis, w/o Myo, wo Rad</u> M47.811 OAA M3 M47.812 Cervical M3 M47.813 CT M3 <u>Other Spondylosis w Rad</u> M47.21 OAA M3 M47.22 Cervical M3 M47.23 CT M3 <u>Other Spondylosis</u> M47.891 OAA M3 M47.892 Cervical M3 M47.893 CT M3 M79.12 Myalgia head and neck muscles M3 M79.7 Fibromyalgia M3 <u>Sprain/Strain</u> S13.4XXA Sprain (C0-C2)/Whiplash M1 S13.8XXA Sprain (C2-C7) M3 S16.1XXA Strain M3 M43.6 Torticollis M4 <u>Spinal Stenosis</u> M48.01 OAA M4 M48.02 Cervical M4 M48.03 CT M4 M53.0 Cervicocranial Syndrome M4 M53.1 Cervicobrachial Syndrome M4 <u>Radiculitis/Neuritis</u> M54.11 Radiculopathy, OAA M4 M54.12 Radiculopathy, Cervical M4 M54.13 Radiculopathy, CT M4 <u>Disc Disorder w Rad</u> M50.11 High Cervical (C2, C3, C4) M4 M50.12 + Mid Cervical M4 M50.13 CT (C8) M4 <u>Other Disc Displacement</u> M50.21 High Cervical (C2-3,3-4) M3 M50.22 + Mid Cervical M3 M50.23 CT (C7-T1) M3 S13.0XXA Traumatic Rupture IVD <u>Disc Degeneration</u> M50.31 High Cervical (C2-3,3-4) M3 M50.32 + Mid Cervical M3 M50.33 CT (C7-T1) M3 + <u>Mid Cervical Level:</u> 0 = Unspecified 1 = C4-C5 2 = C5-C6 3 = C6-C7	M99.02 Segmental Dysfunction M1 M54.6 Pain in the Thoracic Spine M1 M62.830 Muscle Spasm of Back M2 <u>Spondylosis, wo Myo, wo Rad</u> M47.814 Thoracic M3 M47.815 TL M3 <u>Other Spondylosis, w Rad</u> M47.24 Thoracic M3 M47.25 TL M3 <u>Other Spondylosis</u> M47.894 Thoracic M3 M47.895 TL M3 M79.18 Myalgia other site M3 M79.7 Fibromyalgia M3 <u>Sprain/Strain</u> S23.3XXA Sprain Thoracic M2 S23.8XXA Sprain Other Thorax M3 <u>Disc Disorder w Rad</u> M51.14 Thoracic M3 M51.15 TL M3 <u>Other Disc Displacement</u> M51.24 Thoracic M3 M51.25 TL M3 S23.0XXA Traumatic Rupture IVD M3 <u>Disc Degeneration</u> M51.34 Thoracic M3 M51.35 TL M3 <u>Spinal Stenosis</u> M48.04 Thoracic M3 M48.05 TL M3 <u>Radiculitis/Neuritis</u> M54.14 Radiculopathy, Thoracic M3 M54.15 Radiculopathy, TL M3	M99.03 Segmental Dysfunction M1 M54.5 Lumbago M2 M62.830 Muscle Spasm of Back M2 M79.18 Myalgia other site M3 M79.7 Fibromyalgia M3 <u>Sprain/Strain</u> S33.5XXA Sprain Lumbar M3 S39.012A Strain Lumbar S33.8XXA Sp. Other Lumbar/Pelvis M3 <u>Spondylosis, wo Myo, wo Rad</u> M47.816 Lumbar M3 M47.817 Lumbosacral M3 <u>Other Spondylosis w Rad</u> M47.26 Lumbar M3 M47.27 Lumbosacral M3 <u>Other Spondylosis</u> M47.896 Lumbar M3 M47.897 Lumbosacral M3 <u>Disc Disorder w Rad</u> M51.16 Lumbar M3 M51.17 Lumbosacral M3 <u>Other Disc Displacement</u> M51.26 Lumbar M3 M51.27 Lumbosacral M3 S33.0XXA Traumatic Rupture IVD M3 <u>Disc Degeneration</u> M51.36 Lumbar M3 M51.37 Lumbosacral M3 <u>Spinal Stenosis</u> M48.061 Lumbar w Neuro Claud M3 M48.062 Lumbar w Neuro Claud M3 M48.07 Lumbosacral M3 <u>Sciatica</u> M54.41 Lumbago w Sciatica- R M3 M54.42 Lumbago w Sciatica- L M3 <u>Radiculitis/Neuritis</u> M54.16 Lumbar M3 M54.17 Lumbosacral M3 <u>Spondylolisthesis, Acq.</u> M43.16 Lumbar M3 M43.17 Lumbosacral M3	M99.04 Segmental Dysfunction - Sacral M1 M99.05 Segmental Dysfunction - Pelvic M1 M62.830 Muscle Spasm of Back M2 M79.18 Myalgia other site M3 M79.7 Fibromyalgia M3 <u>Sprain/Strain</u> S33.6XXA Sprain SI Joint M3 S33.8XXA Sacrum M3 S33.8XXA Sp. Other Lumbar/Pelvis M3 S39.013A Strain Pelvis <u>Spondylosis, wo Myo, wo Rad</u> M47.818 Sacral/Sacroccocygeal M3 <u>Other Spondylosis w Rad</u> M47.28 Sacral/Sacroccocygeal M3 <u>Other Spondylosis</u> M47.898 Sacral/Sacroccocygeal M3 <u>Sciatica</u> M54.31 Sciatica- R M3 M54.32 Sciatica - L M3 M53.3 Sacrum Disorders M3 M53.3 Coccyx Disorders M3
LOWER EXTREMITIES			
M99.06 Segmental Dysfunction - LE			
Hip: M25.55* Hip Joint Pain S73.11*A Sprain - Iliofemoral Lig S73.12*A Sprain - Ischiocapsular Lig S73.19*A Sprain - Other			
Knee: M25.56* Knee Joint Pain S83.41*A Sprain - MCL S83.42*A Sprain - LCL S83.51*A Sprain - ACL S83.52*A Sprain - PCL S83.6*X A Sprain - Sup Tib/Fib Jt S83.8X*A Sprain - Other			
Ankle/Foot: M79.67* Ankle/Foot Joint Pain S93.41*A Sprain - Calcaneofibular Lig S93.42*A Sprain - Deltoid Lig S93.43*A Sprain - Tibiofibular Lig S93.49*A Sprain - Other Lig S93.61*A Sprain - Tarsal Lig S93.62*A Sprain - Tarsometatarsal Lig S93.69*A Sprain - Other Lig			
* Laterality: 1 = Right 2 = Left 9 = Unspecified			
Physical Examination (Continued)			
Chest (Breast)			
Ears, Nose, Mouth, & Throat			
Eyes			
Gastrointestinal (Abdomen)			
Genitourinary			
Neck			
Respiratory			

Functional Rating Index

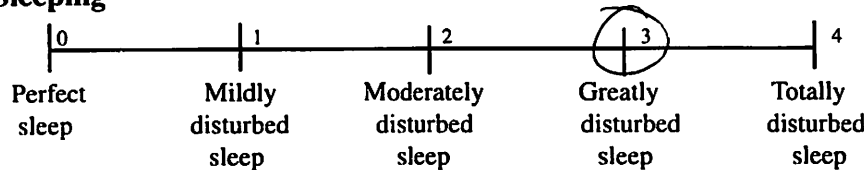
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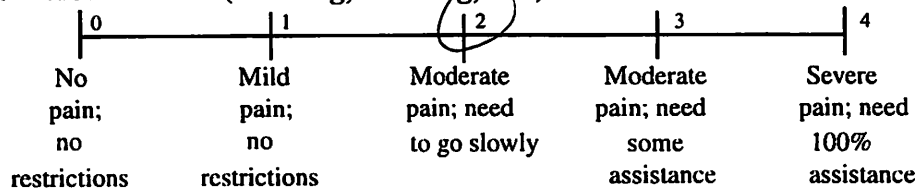
1. Pain Intensity



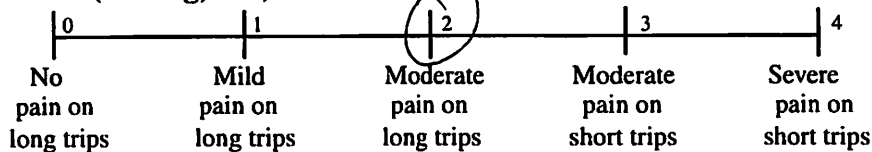
2. Sleeping



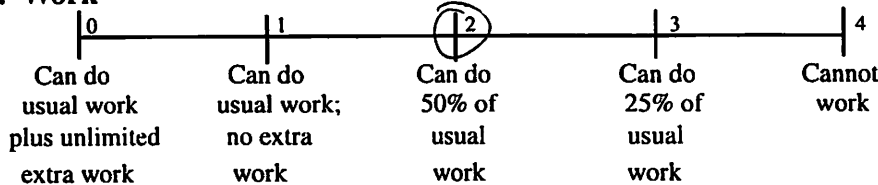
3. Personal Care (washing, dressing, etc.)



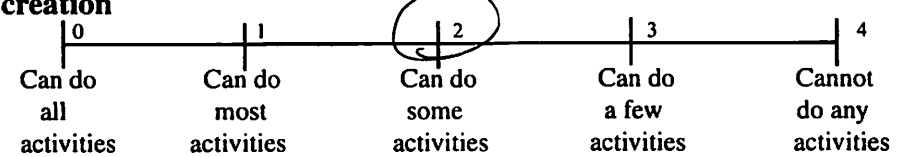
4. Travel (driving, etc.)



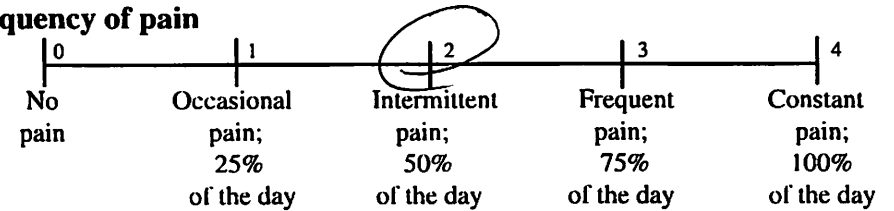
5. Work



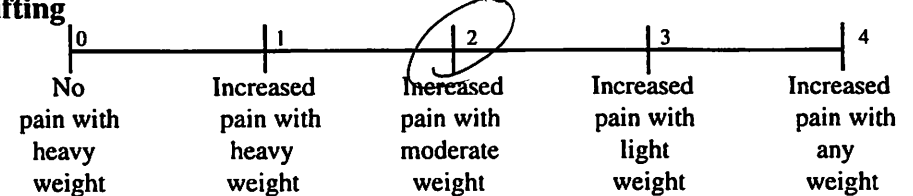
6. Recreation



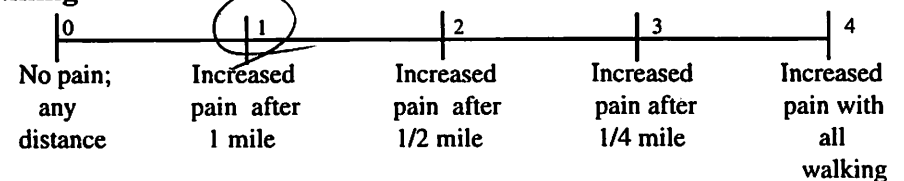
7. Frequency of pain



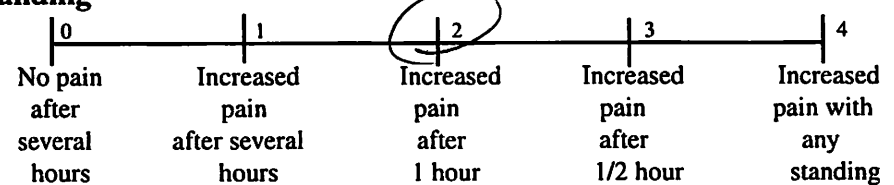
8. Lifting



9. Walking



10. Standing



Name Jeff Plummer

PRINTED

Signature

ID#/SS#

Plan ID

Total Score

Date

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DAILY NOTE

Encounter Date: 1/14/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

Rendering Physician: Heidi J. Pickering, DC

DOB: [REDACTED]

Individual NPI: 1356643423

Account No: 1042

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. The patient presents to the office today for an initial consultation and evaluation for their condition. They describe the severity of today's complaints as severe pain VRS (9-10/10 NRS). Jeffery states he was traveling at the intersection of Tarpon Rd and US 19. He was looking to his left when he was struck from the back by the other vehicle. Immediately following the accident he has been experiencing pain that has been progressively getting worse.

He states he has been having difficulty sleeping since the accident. He has been experiencing changes in his bowels since the accident.

Patient's major complaint is stiff sore neck. He reports it began 1/5/2021 The patient states it started car accident. Daily activities being affected by this condition: all depending on situation . The patient reports his pain frequency is off and on. Patient reports the quality of pain is sharp stiff and sore. The patient reports his current complaint radiates/shoots into the arms. The patient reports that their complaint improves with movement stretching. The patient reports that their complaint worsens with laying down/sleeping. Previous treatment reported for this condition: none. Patient reports the following previous diagnostic testing has been performed for this condition: none.

MEDICATIONS:

The patient reported they are currently not taking any medications.

ALLERGIES to MEDICATIONS:

No known drug allergies

PAST HEALTH HISTORY:

Prior Injuries, Traumas, and Hospitalizations:

Injuries:

broken bones

Prior Surgeries:

No surgical intervention was reported.

FAMILY HEALTH HISTORY:

Relevant major health problems of first degree relatives: Unknown

SOCIAL AND OCCUPATIONAL HISTORY:

Marital status: single

Children: 1

Student status: non-student

Level of education: high school

Employed: yes

Dominant hand: right

Alcohol: occasionally

Caffeine: coffee

Exercise: 2-3xs a week

Smoking Status: Never smoker: an individual who has not smoke 100 or more cigarettes during his/her lifetime

REVIEW OF SYSTEMS:

Constitutional-

Complains of: fatigue

Musculoskeletal-

Denies: broken bones

Complains of: as stated in the history of present illness

Neurological-

Denies: dizziness or light headed, convulsions or seizures, tremors

Complains of: as stated in the history of present illness

Psychiatric-

Denies: nervousness/anxiety, depression, sleep problems, memory loss or confusion

Genitourinary-

Denies: frequent or painful urination, blood in urine, incontinence or bed wetting, painful or irregular periods

Gastrointestinal-

Denies: loss of appetite, blood in stool or black stool, nausea or vomiting, abdominal pain, constipation

Complains of: frequent diarrhea,

Cardiovascular & heart-

Denies: chest pains/tightness, rapid or heartbeat changes, swelling of hands, ankles or feet

Respiratory-

Denies: difficulty breathing, cough

Eyes & vision-

Denies: eye pain, blurred or double vision, sensitivity to light

Head, ears, nose, mouth/throat-

Denies: frequent or recurrent headaches, ear - ache/ringing/drainage, hearing loss, sensitivity to loud noises, sinus problems, sore throat

Endocrine-

Denies: infertility, recent weight change, eating disorder

Hematologic & lymphatic-

Denies: excessive thirst or urination, cold extremities, swollen glands

Integumentary-

Denies: rash or itching, change in skin, hair, or nails, non-healing sores or lesions, change of appearance of a mole, breast pain, lump, or discharge

Allergic/immunologic-

Denies: food allergies, environmental allergies

Objective:

Palpatory tenderness (4/4), spasms (grade 5/5), hypomobility and asymmetry are a severe level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region.

Vitals:

Height: 72 "

Total Weight: 181 lbs.

BMI: 24.5 kg/m²

Left Blood Pressure: 137/86 mmHg

Pulse: 76 bpm

Appearance: no apparent distress

Mood and Affect: normal

Orientation: alert and orientated to time, place, and person

Gait and Station:

gait - visibly normal

station - abnormal as documented in bilateral weight- LT: 83 lbs RT: 98 lbs

Neurological:

DTRs-

biceps: LT= 2+ RT= 2+

brachioradialis: LT= 2+ RT= 2+

triceps: LT= 2+ RT= 2+

Cardiovascular:

Exam of the upper extremities is unremarkable.

Lymphatic:

Exam of the head/neck and upper extremities is unremarkable.

Skin:

Exam of the head/neck, trunk, and upper extremities is unremarkable.

Musculoskeletal:

Cervical Spine

	Flexion	40/50
	Extension	53/60
LT 70/80	Rotation	73/80 RT
LT 32/45	Lateral Bending	40/45 RT

Dorso-Lumbar Spine

	Flexion	43/60
	Extension	20/25
LT 7/25	Rotation	20/25 RT
LT 15/25	Lateral Bending	23/25 RT

- + Compression Test on the left
- + Compression Test on the right
- + Kemp's Test on the left
- + Kemp's Test on the right

Slight curvature/towering noted on thoracic spine x-rays.

Misalignments correlate with patients complaints.

Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 21-40% range of disability on the FRI estimate of disability scale. This corresponds with a MODERATE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. The current FRI estimated disability score is 27.5%

The specific findings of the FRI include:

- Pain Intensity - (2) Moderate pain;
- Sleeping - (3) Greatly Disturbed sleep;
- Personal Care - (1) Mild pain with no restrictions;
- Travel - (0) No pain on long trips;
- Work - (1) Can do usual work, but no extra work;
- Recreation - (1) Can do most activities;
- Frequency of Pain - (2) Intermittent Pain: 50% of the day;

Lifting - (0) No pain with heavy weight;
Walking - (0) No pain at any distance;
Standing - (1) Increased pain after several hours;

Results:

X-ray Lumbar

Misalignments in area of complaint. Findings correlate with complaint level.

Do's and Dont's

Patient is to return on next office day for treatment plan.

Thoracic x-rays

Misalignments in area of complaint. Findings correlate with complaint level.

Cervical X-ray

Misalignments in area of complaint. Findings correlate with complaint level.

Assessment:

The patient tolerated today's evaluation and treatments without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L5R, L4L

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

A new patient examination was performed today. X-rays taken were anterior to posterior, and lateral cervical spine views. X-rays taken were anterior to posterior, and lateral thoracic spine views. X-rays taken were anterior to posterior, and lateral lumbar spine views. Chiropractic spinal manipulation was administered to reduce subluxations. Manual manipulation performed at: C2, C6, T6, T7, L5, L4

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020

Resolution of Chief Complaint: Approximately 90 days.

Progress Evaluation: Re-evaluation in approximately 30 days.

Frequency: 3 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.

Orders:

Instructions: Do's and Dont's

I have discussed standard "dos and don'ts" and standard home care recommendations that are consistent with the patient's symptoms. The patient has been instructed not to vacuum or sweep, soak in a hot tub, or lift over 10 pounds until I see them next. When tolerable to the patient, I will be instructing the patient on a home based rehabilitative exercise regimen designed to improve function and overall muscular strength.

Radiology: Lumbar x-ray

X-rays ordered were anterior to posterior and lateral lumbar views.

Radiology: Thoracic x-ray

X-rays ordered of anterior to posterior, and lateral thoracic spine views.

Radiology: Cervical x-ray

Cervical x-rays taken anterior to posterior, AP open mouth, and lateral.



Electronically signed by: Heidi J. Pickering, DC

1/14/2021 5:46 PM

DAILY NOTE

Encounter Date: 1/18/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in severe pain VRS (9-10/10 NRS).

Objective:

Palpatory tenderness (4/4), spasms (5/5), hypomobility, and asymmetry continue to be at a severe level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 1%-20%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Exercises are provided today to enhance chiropractic care by restoring range of motion and strengthening stabilizing muscles. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
1/18/2021 10:13 AM

DAILY NOTE

Encounter Date: 1/19/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in severe pain VRS (9-10/10 NRS).

Objective:

Palpatory tenderness (4/4), spasms (5/5), hypomobility, and asymmetry continue to be at a severe level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The condition of the patient is unchanged from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 1%-20%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

1/19/2021 10:55 AM

DAILY NOTE

Encounter Date: 1/21/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in severe pain VRS (9-10/10 NRS).

Objective:

Palpatory tenderness (4/4), spasms (5/5), hypomobility, and asymmetry continue to be at a severe level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 1%-20%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

1/21/2021 11:24 AM

DAILY NOTE

Encounter Date: 1/25/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in severe pain VRS (9-10/10 NRS).

Objective:

Palpatory tenderness (4/4), spasms (5/5), hypomobility, and asymmetry continue to be at a severe level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition is the same as their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 1%-20%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

1/25/2021 10:28 AM

DAILY NOTE

Encounter Date: 1/26/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: today the patient says they're better than their previous visit. The pain scale has decreased to moderate-severe pain VRS (7-8/10 NRS).

Objective:

Palpatory tenderness (3/4), spasms (grade 4/5), hypomobility, and asymmetry have improved today to a moderate-severe level. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient is better than last visit. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 21%-40%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

1/26/2021 10:15 AM

DAILY NOTE

Encounter Date: 1/28/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate-severe pain VRS (7-8/10 NRS).

Objective:

Palpatory tenderness (3/4), spasms (grade 4/5), hypomobility, and asymmetry continue to be at a moderate-severe level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain the same as their previous treatment. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 21%-40%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

1/28/2021 10:24 AM

DAILY NOTE

Encounter Date: 2/1/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: since their last visit the patient complains they are improved. The pain scale has decreased to moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry have improved today to a moderate level. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient is better than last visit. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

2/1/2021 10:29 AM

DAILY NOTE

Encounter Date: 2/2/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.

Orders:

Radiology:

Patient was referred for a lumbar spine MRI. Patient was referred for a cervical spine MRI.



Electronically signed by: Heidi J. Pickering, DC

2/2/2021 10:47 AM

DAILY NOTE

Encounter Date: 2/4/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

2/4/2021 11:19 AM

DAILY NOTE

Encounter Date: 2/8/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

2/8/2021 10:05 AM

DAILY NOTE

Encounter Date: 2/10/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Patient complains today of muscle spasms along both sides of the spine. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been no change in the patient's condition from their previous visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

2/10/2021 10:30 AM

DAILY NOTE

Encounter Date: 2/11/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Hypertonicity with trigger points noted in cervical, thoracic, and lumbar spinal musculature. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
2/11/2021 9:53 AM

DAILY NOTE

Encounter Date: 2/15/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. The patient rates their pain as moderate pain VRS (5-6/10 NRS) today.

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry are a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 21-40% range of disability on the FRI estimate of disability scale. This corresponds with a MODERATE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 35% (14)

The specific findings of the FRI are:

~Pain Intensity: 2 - Moderate pain

~Sleeping: 3 -Greatly disturbed sleep

~Personal Care (washing, dressing, etc.): 1 - Mild pain; no restrictions
~Travel (driving, etc.): 0 - No pain on long trips
~Work: 1 - Can do usual work; no extra work
~Recreation: 2 - Can do some activities
~Frequency of Pain: 2 - Intermittent pain; 50% of the day
~Lifting: 2 - Increased pain with moderate weight
~Walking: 0 - No pain; any distance
~Standing: 1 - Increased pain after several hours

@ 1999-2001 Institute of Evidence-Based Chiropractic

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020

Resolution of Chief Complaint: Approximately 90 days.

Progress Evaluation: Re-evaluation in approximately 30 days.

Frequency: 3 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.

Orders:

Referrals:

Patient needs a orthopedic evaluation and treatment.

Elite Spine & Ortho

Trintiy Location
2040 Short Ave.
Odessa FL 33556
(855) 77-SPINE

Tampa Location
5421 Beaumont Center Blvd. Suite # 620
Tampa FL 33634
(855) 77-SPINE



Electronically signed by: Heidi J. Pickering, DC

2/15/2021 10:50 AM

DAILY NOTE

Encounter Date: 2/16/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.

Orders:

Referrals: Ortho

Patient needs a orthopedic evaluation and treatment.

Elite Spine & Ortho

Trintiy Location

2040 Short Ave.

Odessa FL 33556

(855) 77-SPINE

Tampa Location

5421 Beaumont Center Blvd. Suite # 620

Tampa FL 33634

(855) 77-SPINE



Electronically signed by: Heidi J. Pickering, DC

2/16/2021 9:52 AM

DAILY NOTE

Encounter Date: 2/18/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

Rendering Physician: Heidi J. Pickering, DC

DOB: [REDACTED]

Individual NPI: 1356643423

Account No: 1042

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
2/18/2021 10:09 AM

DAILY NOTE

Encounter Date: 2/22/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
2/22/2021 10:23 AM

DAILY NOTE

Encounter Date: 2/23/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
2/23/2021 10:13 AM

DAILY NOTE

Encounter Date: 2/25/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The condition of the patient is unchanged from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
2/25/2021 2:51 PM

DAILY NOTE

Encounter Date: 3/1/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been no change in the patient's condition from their previous visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

3/1/2021 10:14 AM

DAILY NOTE

Encounter Date: 3/2/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The condition of the patient is unchanged from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/2/2021 10:12 AM

DAILY NOTE

Encounter Date: 3/4/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/4/2021 12:40 PM

DAILY NOTE

Encounter Date: 3/8/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been no change in the patient's condition from their previous visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/8/2021 10:21 AM

DAILY NOTE

Encounter Date: 3/9/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/9/2021 10:12 AM

DAILY NOTE

Encounter Date: 3/11/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/11/2021 10:10 AM

DAILY NOTE

Encounter Date: 3/15/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/15/2021 11:04 AM

DAILY NOTE

Encounter Date: 3/16/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been no change in the patient's condition from their previous visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/16/2021 11:06 AM

DAILY NOTE

Encounter Date: 3/17/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left radiating around the left rib cage. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/17/2021 11:39 AM

DAILY NOTE

Encounter Date: 3/22/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. The patient rates their pain as moderate pain VRS (5-6/10 NRS) today.

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry are a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

- kemps

+compression on the right

+compression on the left

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 21-40% range of disability on the FRI estimate of disability scale. This corresponds with a MODERATE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 40% (16)

The specific findings of the FRI are:

- ~Pain Intensity: 2 - Moderate pain
- ~Sleeping: 3 -Greatly disturbed sleep
- ~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly
- ~Travel (driving, etc.): 3 - Moderate pain on short trips
- ~Work: 1 - Can do usual work; no extra work
- ~Recreation: 1 - Can do most activities
- ~Frequency of Pain: 1 - Occasional pain; 25% of the day
- ~Lifting: 1 - Increased pain with heavy weight
- ~Walking: 1 - Increased pain after 1 mile
- ~Standing: 1 - Increased pain after several hours

@ 1999-2001 Institute of Evidence-Based Chiropractic

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

- S13.4XXA - Sprain of ligaments of cervical spine, initial encounter
- S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter
- S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter
- M54.11 - Radiculopathy, occipito-atlanto-axial region
- M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020

Resolution of Chief Complaint: Approximately 90 days.

Progress Evaluation: Re-evaluation in approximately 30 days.

Frequency: 2 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.



Electronically signed by: Heidi J. Pickering, DC

3/22/2021 12:17 PM

DAILY NOTE

Encounter Date: 3/23/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

Rendering Physician: Heidi J. Pickering, DC

DOB: [REDACTED]

Individual NPI: 1356643423

Account No: 1042

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Since the previous visit the Patient's condition is unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/23/2021 11:39 AM

DAILY NOTE

Encounter Date: 3/25/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The condition of the patient is the same as their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/25/2021 11:54 AM

DAILY NOTE

Encounter Date: 3/29/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/29/2021 10:05 AM

DAILY NOTE

Encounter Date: 3/30/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The condition of the patient is unchanged from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
3/30/2021 11:23 AM

DAILY NOTE

Encounter Date: 4/1/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
4/1/2021 10:34 AM

DAILY NOTE

Encounter Date: 4/5/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain the same as their previous treatment. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
4/5/2021 10:45 AM

DAILY NOTE

Encounter Date: 4/7/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
4/7/2021 10:21 AM

DAILY NOTE

Encounter Date: 4/12/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
4/12/2021 11:30 AM

DAILY NOTE

Encounter Date: 4/14/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
4/14/2021 10:23 AM

DAILY NOTE

Encounter Date: 4/19/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

Rendering Physician: Heidi J. Pickering, DC

DOB: [REDACTED]

Individual NPI: 1356643423

Account No: 1042

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Since the previous visit the Patient's condition is unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
4/19/2021 10:29 AM

DAILY NOTE

Encounter Date: 4/21/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

Rendering Physician: Heidi J. Pickering, DC

DOB: [REDACTED]

Individual NPI: 1356643423

Account No: 1042

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. The patient rates their pain as moderate pain VRS (5-6/10 NRS) today.

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry are a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

+compression on the left

+compression on the right

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 41-60% range of disability on the FRI estimate of disability scale. This corresponds with a SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 48% (19)

The specific findings of the FRI are:

- ~Pain Intensity: 2 - Moderate pain
- ~Sleeping: 3 -Greatly disturbed sleep
- ~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly
- ~Travel (driving, etc.): 2 - Moderate pain on long trips
- ~Work: 1 - Can do usual work; no extra work
- ~Recreation: 2 - Can do some activities
- ~Frequency of Pain: 2 - Intermittent pain; 50% of the day
- ~Lifting: 2 - Increased pain with moderate weight
- ~Walking: 1 - Increased pain after 1 mile
- ~Standing: 2 - Increased pain after 1 hour

@ 1999-2001 Institute of Evidence-Based Chiropractic

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

- S13.4XXA - Sprain of ligaments of cervical spine, initial encounter
- S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter
- S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter
- M54.11 - Radiculopathy, occipito-atlanto-axial region
- M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020

Resolution of Chief Complaint: Approximately 90 days.

Progress Evaluation: Re-evaluation in approximately 30 days.

Frequency: 2 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.



Electronically signed by: Heidi J. Pickering, DC

4/21/2021 10:04 AM

DAILY NOTE

Encounter Date: 4/26/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

Rendering Physician: Heidi J. Pickering, DC

DOB: [REDACTED]

Individual NPI: 1356643423

Account No: 1042

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been no change in the patient's condition from their previous visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
4/26/2021 10:46 AM

DAILY NOTE

Encounter Date: 4/28/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
4/28/2021 10:38 AM

DAILY NOTE

Encounter Date: 5/3/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
5/3/2021 11:30 AM

DAILY NOTE

Encounter Date: 5/5/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
5/5/2021 12:07 PM

DAILY NOTE

Encounter Date: 5/10/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain the same as their previous treatment. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
5/10/2021 11:33 AM

DAILY NOTE

Encounter Date: 5/12/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
5/12/2021 11:36 AM

DAILY NOTE

Encounter Date: 5/17/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
5/17/2021 11:58 AM

DAILY NOTE

Encounter Date: 5/19/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
5/19/2021 11:04 AM

DAILY NOTE

Encounter Date: 5/24/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. The patient rates their pain as moderate pain VRS (5-6/10 NRS) today.

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry are a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

+compression on the left and right

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 41-60% range of disability on the FRI estimate of disability scale. This corresponds with a SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 48% (19)

The specific findings of the FRI are:

~Pain Intensity: 2 - Moderate pain
~Sleeping: 3 -Greatly disturbed sleep
~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly
~Travel (driving, etc.): 2 - Moderate pain on long trips
~Work: 1 - Can do usual work; no extra work
~Recreation: 2 - Can do some activities
~Frequency of Pain: 2 - Intermittent pain; 50% of the day
~Lifting: 2 - Increased pain with moderate weight
~Walking: 1 - Increased pain after 1 mile
~Standing: 2 - Increased pain after 1 hour

@ 1999-2001 Institute of Evidence-Based Chiropractic

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter
S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter
S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter
M54.11 - Radiculopathy, occipito-atlanto-axial region
M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020
Resolution of Chief Complaint: Approximately 90 days.
Progress Evaluation: Re-evaluation in approximately 30 days.
Frequency: 2 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.



Electronically signed by: Heidi J. Pickering, DC

5/24/2021 11:47 AM

DAILY NOTE

Encounter Date: 5/26/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

Rendering Physician: Heidi J. Pickering, DC

DOB: [REDACTED]

Individual NPI: 1356643423

Account No: 1042

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
5/26/2021 11:06 AM

DAILY NOTE

Encounter Date: 6/2/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The condition of the patient is unchanged from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
6/2/2021 11:14 AM

DAILY NOTE

Encounter Date: 6/3/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Patient complains of low back pain on the left side. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at L4 on the left is identified via static palpation. Motion palpation of L4 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower left lumbar musculature. Global restriction of the lower lumbar spinal region. Asymmetry at L5 on the right is identified via static palpation. Motion palpation of L5 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right lumbar musculature. Global restriction of the lower lumbar spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R, L4L, L5R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7, L4, L5

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
6/3/2021 11:14 AM

DAILY NOTE

Encounter Date: 6/7/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

6/7/2021 9:46 AM

DAILY NOTE

Encounter Date: 6/9/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

6/9/2021 11:23 AM

DAILY NOTE

Encounter Date: 6/10/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

6/10/2021 10:56 AM

DAILY NOTE

Encounter Date: 6/21/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

There has been no change in the patient's condition from their previous visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

6/21/2021 10:54 AM

DAILY NOTE

Encounter Date: 6/23/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. The patient rates their pain as moderate pain VRS (5-6/10 NRS) today.

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry are a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 41-60% range of disability on the FRI estimate of disability scale. This corresponds with a SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 45% (18)

The specific findings of the FRI are:

- ~Pain Intensity: 2 - Moderate pain
- ~Sleeping: 3 -Greatly disturbed sleep
- ~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly
- ~Travel (driving, etc.): 2 - Moderate pain on long trips
- ~Work: 1 - Can do usual work; no extra work
- ~Recreation: 2 - Can do some activities
- ~Frequency of Pain: 2 - Intermittent pain; 50% of the day
- ~Lifting: 2 - Increased pain with moderate weight

~Walking: 1 - Increased pain after 1 mile
~Standing: 1 - Increased pain after several hours

@ 1999-2001 Institute of Evidence-Based Chiropractic

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6, T6, T7

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020

Resolution of Chief Complaint: Approximately 90 days.

Progress Evaluation: Re-evaluation in approximately 30 days.

Frequency: 1 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.



Electronically signed by: Heidi J. Pickering, DC

6/23/2021 10:46 AM

DAILY NOTE

Encounter Date: 6/29/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

6/29/2021 11:10 AM

DAILY NOTE

Encounter Date: 6/30/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Since the previous visit the Patient's condition is unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

6/30/2021 10:52 AM

DAILY NOTE

Encounter Date: 7/6/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient's condition is the same as their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

7/6/2021 10:31 AM

DAILY NOTE

Encounter Date: 7/8/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient's condition is the same as their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

7/8/2021 11:18 AM

DAILY NOTE

Encounter Date: 7/15/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Since the previous visit the Patient's condition is unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

7/15/2021 10:57 AM

DAILY NOTE

Encounter Date: 7/20/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

7/20/2021 11:02 AM

DAILY NOTE

Encounter Date: 7/22/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

The condition of the patient is unchanged from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

7/22/2021 10:53 AM

DAILY NOTE

Encounter Date: 7/27/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. The patient rates their pain as moderate pain VRS (5-6/10 NRS) today.

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry are a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

+compression on the left and right

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 41-60% range of disability on the FRI estimate of disability scale. This corresponds with a SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 48% (19)

The specific findings of the FRI are:

~Pain Intensity: 2 - Moderate pain

~Sleeping: 3 -Greatly disturbed sleep

~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly

~Travel (driving, etc.): 3 - Moderate pain on short trips

~Work: 1 - Can do usual work; no extra work

~Recreation: 2 - Can do some activities

~Frequency of Pain: 2 - Intermittent pain; 50% of the day
~Lifting: 2 - Increased pain with moderate weight
~Walking: 1 - Increased pain after 1 mile
~Standing: 1 - Increased pain after several hours

@ 1999-2001 Institute of Evidence-Based Chiropractic

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6, T6, T7

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020

Resolution of Chief Complaint: Approximately 90 days.

Progress Evaluation: Re-evaluation in approximately 30 days.

Frequency: 2 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.



Electronically signed by: Heidi J. Pickering, DC

7/27/2021 11:01 AM

DAILY NOTE

Encounter Date: 7/29/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient's condition is the same as their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

7/29/2021 12:03 PM

DAILY NOTE

Encounter Date: 8/4/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

8/4/2021 2:48 PM

DAILY NOTE

Encounter Date: 8/5/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

8/5/2021 10:14 AM

DAILY NOTE

Encounter Date: 8/11/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Since the previous visit the Patient's condition is unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

8/11/2021 10:24 AM

DAILY NOTE

Encounter Date: 8/12/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

8/12/2021 9:17 AM

DAILY NOTE

Encounter Date: 8/18/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

8/18/2021 10:07 AM

DAILY NOTE

Encounter Date: 8/19/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

The condition of the patient is unchanged from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

8/19/2021 11:10 AM

DAILY NOTE

Encounter Date: 8/24/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient's condition continues to remain unchanged from their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation.

Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

8/24/2021 11:02 AM

DAILY NOTE

Encounter Date: 8/26/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of a headache. Patient complains of midback pain on the left. The patient rates their pain as moderate pain VRS (5-6/10 NRS) today.

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry are a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
8/26/2021 10:12 AM

DAILY NOTE

Encounter Date: 8/31/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of midback pain on the left. The patient rates their pain as moderate pain VRS (5-6/10 NRS) today.

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry are a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

+compression on the right and left

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 41-60% range of disability on the FRI estimate of disability scale. This corresponds with a SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 52% (21)

The specific findings of the FRI are:

~Pain Intensity: 2 - Moderate pain

~Sleeping: 3 -Greatly disturbed sleep

~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly

~Travel (driving, etc.): 2 - Moderate pain on long trips

~Work: 2 - Can do 50% of usual work

~Recreation: 2 - Can do some activities

~Frequency of Pain: 2 - Intermittent pain; 50% of the day
~Lifting: 2 - Increased pain with moderate weight
~Walking: 2 - Increased pain after 1/2 mile
~Standing: 2 - Increased pain after 1 hour

@ 1999-2001 Institute of Evidence-Based Chiropractic

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6, T6, T7

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020

Resolution of Chief Complaint: Approximately 90 days.

Progress Evaluation: Re-evaluation in approximately 30 days.

Frequency: 1 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.



Electronically signed by: Heidi J. Pickering, DC

8/31/2021 11:12 AM

DAILY NOTE

Encounter Date: 9/1/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

The condition of the patient is unchanged from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
9/1/2021 10:57 AM

DAILY NOTE

Encounter Date: 9/14/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Since the previous visit the Patient's condition is unchanged. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
9/14/2021 10:21 AM

DAILY NOTE

Encounter Date: 9/16/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in moderate pain VRS (5-6/10 NRS).

Objective:

Palpatory tenderness (2/4), spasms (grade 3/5), hypomobility, and asymmetry continue to be at a moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

There has been little change in the patient's condition from their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 41%-60%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
9/16/2021 11:15 AM

DAILY NOTE

Encounter Date: 9/21/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of midback pain on the left. Changes since last visit: patient stated today they are better than their previous visit. The pain scale has decreased to mild-moderate pain VRS (3-4/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 2/5), hypomobility, and asymmetry have improved today to a mild-moderate level. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient is better than last visit. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 61%-80%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
9/21/2021 11:30 AM

DAILY NOTE

Encounter Date: 9/23/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in mild-moderate pain VRS (3-4/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 2/5), hypomobility, and asymmetry continue to be at a mild-moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

Patient's condition is the same as their last visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 61%-80%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
9/23/2021 10:52 AM

DAILY NOTE

Encounter Date: 9/28/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side radiating to left arm to hand. Patient complains of neck pain on the right side radiating to right arm to right hand. Patient complains of midback pain on the left. Changes since last visit: Patient reports there are no changes since their last visit. They remain in mild-moderate pain VRS (3-4/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 2/5), hypomobility, and asymmetry continue to be at a mild-moderate level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. On palpation, pain and swelling is noted in the upper thoracic spine. There is restricted intersegmental motion due to pain on palpation. Muscle spasms are also noted in the upper dorsal paraspinal musculature as well as the lower trapezius musculature. Tenderness upon palpation of cervicothoracic region. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

Assessment:

The condition of the patient is the same as their previous visit. Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 61%-80%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

M54.13 - Radiculopathy, cervicothoracic region

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6, T6, T7

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC
9/28/2021 12:19 PM

DAILY NOTE

Encounter Date: 10/6/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: patient stated today they are better than their previous visit. The pain scale has decreased to mild pain VRS (1-2/10 NRS). Patient states that occasionally when he is laying in bed at night his arms will go numb.

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry have improved today to a mild level. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region. Asymmetry at T6 on the right is identified via static palpation. Motion palpation of T6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region. Asymmetry at T7 on the right is identified via static palpation. Motion palpation of T7 reveals hypomobility at this level. Taut and tender muscle fibers are present in the mid right thoracic musculature. Global restriction of the mid thoracic spinal region.

- Compression Test on the left
- Compression Test on the right

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 41-60% range of disability on the FRI estimate of disability scale. This corresponds with a SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 48% (19)

The specific findings of the FRI are:

- ~Pain Intensity: 2 - Moderate pain
- ~Sleeping: 3 -Greatly disturbed sleep
- ~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly
- ~Travel (driving, etc.): 2 - Moderate pain on long trips
- ~Work: 2 - Can do 50% of usual work
- ~Recreation: 2 - Can do some activities

~Frequency of Pain: 2 - Intermittent pain; 50% of the day
~Lifting: 1 - Increased pain with heavy weight
~Walking: 2 - Increased pain after 1/2 mile
~Standing: 1 - Increased pain after several hours

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Assessment:

Patient is better than last visit. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R, T6R, T7R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6, T6, T7

PLAN OF CARE:

Initial Date of Treatment: 1/14/2020

Resolution of Chief Complaint: Approximately 90 days.

Progress Evaluation: Re-evaluation in approximately 30 days.

Frequency: 1 times a week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized.

Procedures: Please be advised that the specific procedures listed below are medically necessary based on this patient's history, examination, and diagnosis, and are needed to ensure effective care. This patient has a significant neuromuscular condition and it is my expectation that this patient will benefit from full recovery or at least improve function. The patient has consented to these procedures and plan of care.

-Chiropractic manipulative treatment will be administered to the spinal region(s) due to findings noted in the most recent examination of segmental dysfunction. This will be performed to influence joint and neurophysiological function on each visit.

-Intersegmental mechanical traction to mobilize the restrictions above and below the subluxation.

Patient Instructions: Home exercises will be prescribed at the clinically appropriate time.

Future Tests: No tests are anticipated to be ordered during this plan of care.

Referrals: No Referrals needed at this time.

Contraindications: No contraindications to treatment are identified at this time.

Complicating factors: No complicating factors have been identified at this time.

SPECIFIC TREATMENT GOALS:

Short Term Goals: To decrease pain intensity and pain frequency by 1 level and improve function by 1 level in the AREA section of the Functional Rating Index OAT.

Long Term Goals: To return patient to a level 1 in the pain intensity, a level 1 in the pain frequency, and return patient to a level 1 in the AREA section of the Functional Rating Index OAT.



Electronically signed by: Heidi J. Pickering, DC

10/6/2021 4:01 PM

DAILY NOTE

Encounter Date: 10/13/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Since the previous visit the Patient's condition is unchanged. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

10/13/2021 11:26 AM

DAILY NOTE

Encounter Date: 10/20/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

10/20/2021 11:16 AM

DAILY NOTE

Encounter Date: 10/27/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Since the previous visit the Patient's condition is unchanged. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

10/27/2021 11:37 AM

DAILY NOTE

Encounter Date: 11/4/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

Rendering Physician: Heidi J. Pickering, DC

DOB: [REDACTED]

Individual NPI: 1356643423

Account No: 1042

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. The patient and I have come to a mutual agreement that they have reached maximum medical improvement.

Objective:

Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

+ Compression Left

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 41-60% range of disability on the FRI estimate of disability scale. This corresponds with a SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 50% (20)

The specific findings of the FRI are:

- ~Pain Intensity: 2 - Moderate pain
- ~Sleeping: 3 -Greatly disturbed sleep
- ~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly
- ~Travel (driving, etc.): 2 - Moderate pain on long trips
- ~Work: 2 - Can do 50% of usual work
- ~Recreation: 2 - Can do some activities
- ~Frequency of Pain: 2 - Intermittent pain; 50% of the day
- ~Lifting: 2 - Increased pain with moderate weight
- ~Walking: 1 - Increased pain after 1 mile
- ~Standing: 2 - Increased pain after 1 hour

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Assessment:

The patient and I have come to a mutual agreement that they have reached maximum medical improvement. Based on the presentation today compared to their first visit, they have improved greatly and compare to how they state they were before the accident. Prognosis: The response to date is overall positive, with a percentage of improvement between 85%-100%. It is my clinical opinion that conservative chiropractic maintenance treatment should continue for the patients overall health. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Patient is recommended to continue maintenance chiropractic treatment plan. Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

PLAN OF CARE:

Initial Date of Treatment: 1/14/21

Resolution of Chief Complaint: Pt has reached MMI

Progress Evaluation: Re-evaluation in approximately 120 days.

Frequency: 1 time per week until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized for maintenance care.

Short term: Continue to stabilize the segments of the spine.

Long term: Prevent future dis-ease of the spine.



Electronically signed by: Heidi J. Pickering, DC

11/4/2021 12:50 PM

DAILY NOTE

Encounter Date: 11/10/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Patient's condition continues to remain the same as their previous treatment. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

11/10/2021 10:33 AM

DAILY NOTE

Encounter Date: 11/17/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Patient's condition is the same as their last visit. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

11/17/2021 11:51 AM

DAILY NOTE

Encounter Date: 12/1/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Patient's condition continues to remain the same as their previous treatment. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

12/1/2021 11:29 AM

DAILY NOTE

Encounter Date: 12/8/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

12/8/2021 11:55 AM

DAILY NOTE

Encounter Date: 12/1/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Patient's condition continues to remain the same as their previous treatment. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

12/1/2021 11:29 AM

DAILY NOTE

Encounter Date: 12/8/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Since their previous treatment the patient's condition has remained unchanged. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

12/8/2021 11:55 AM

DAILY NOTE

Encounter Date: 1/4/2022

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

The patient's condition is still unchanged from their last visit to the office. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

1/4/2022 10:52 AM

DAILY NOTE

Encounter Date: 1/20/2022

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. The patient rates their pain as mild pain VRS (1-2/10 NRS) today.

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry are a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

+ Compression Test on the left

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 41-60% range of disability on the FRI estimate of disability scale. This corresponds with a SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 55% (22)

The specific findings of the FRI are:

- ~Pain Intensity: 3 - Severe pain
- ~Sleeping: 3 -Greatly disturbed sleep
- ~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly
- ~Travel (driving, etc.): 2 - Moderate pain on long trips
- ~Work: 2 - Can do 50% of usual work
- ~Recreation: 2 - Can do some activities
- ~Frequency of Pain: 3 - Frequent pain; 75% of the day
- ~Lifting: 2 - Increased pain with moderate weight
- ~Walking: 1 - Increased pain after 1 mile
- ~Standing: 2 - Increased pain after 1 hour

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Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6

PLAN OF CARE:

Initial Date of Treatment: 1/14/21

Resolution of Chief Complaint: Pt has reached MMI

Progress Evaluation: Re-evaluation in approximately 120 days.

Frequency: Every two weeks until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized for maintenance care.

Short term: Continue to stabilize the segments of the spine.

Long term: Prevent future dis-ease of the spine.



Electronically signed by: Heidi J. Pickering, DC

1/20/2022 12:22 PM

DAILY NOTE

Encounter Date: 2/1/2022

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Patient stated today they are slightly worse than their previous visit. The pain scale has increased slightly, but remains in the to mild pain VRS (1-2/10 NRS) range.

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Patient is slightly worse than last visit. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. Despite the slight worsening today, the response to date is overall positive. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

2/1/2022 11:39 AM

DAILY NOTE

Encounter Date: 2/16/2022

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Patient's condition is the same as their last visit. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

2/16/2022 11:16 AM

DAILY NOTE

Encounter Date: 3/1/2022

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. Changes since last visit: Since the last visit patient states that there are no changes. They remain in mild pain VRS (1-2/10 NRS).

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry continue to be at a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

Assessment:

Patient's condition continues to remain the same as their previous treatment. Prognosis: chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement at 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.



Electronically signed by: Heidi J. Pickering, DC

3/1/2022 3:54 PM

DAILY NOTE

Encounter Date: 3/8/2022

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Subjective:

Patient complains of neck pain on the left side. Patient complains of neck pain on the right side. The patient rates their pain as mild pain VRS (1-2/10 NRS) today.

Objective:

Palpatory tenderness (1/4), spasms (grade 1/5), hypomobility, and asymmetry are a mild level today. Palpation of the cervical spine revealed capsular swelling and elicited a pain reaction. Asymmetry at C2 on the left is identified via static palpation. Motion palpation of C2 reveals hypomobility at this level. Taut and tender muscle fibers are present in the upper left cervical musculature. Global restriction of the upper cervical spinal region. Asymmetry at C6 on the right is identified via static palpation. Motion palpation of C6 reveals hypomobility at this level. Taut and tender muscle fibers are present in the lower right cervical musculature. Global restriction of the lower cervical spinal region.

+Compression Right

+Compression Left

OUTCOME ASSESSMENTS:

-Functional Outcome Assessment: A Functional Outcome Evaluation was performed today using the Functional Rating Index (FRI) tool. The FRI is a standardized assessment questionnaire designed to measure the limitations the current condition places on the patient's ability to perform and manage everyday activities (ADLs). The score from today's Functional Outcome Evaluation falls within the 61+% range of disability on the FRI estimate of disability scale. This corresponds with a VERY SEVERE disability rating due to the patient's identified deficiencies in function and behavior, as well as the patient's perceived symptoms. Goals to improve these affected ADLs, improve function, improve behavior, and reduce symptoms, as identified by the FRI evaluation, are addressed by the current treatment plan. FRI Score: The current FRI estimated disability score is 70% (28)

The specific findings of the FRI are:

~Pain Intensity: 3 - Severe pain

~Sleeping: 3 -Greatly disturbed sleep

~Personal Care (washing, dressing, etc.): 2 - Moderate pain; need to go slowly

~Travel (driving, etc.): 3 - Moderate pain on short trips

~Work: 3 - Can do 25% of usual work

~Recreation: 2 - Can do some activities

~Frequency of Pain: 3 - Frequent pain; 75% of the day

~Lifting: 3 - Increased pain with light weight

~Walking: 3 - Increased pain after 1/4 mile

~Standing: 3 - Increased pain after 1/2 hour

@ 1999-2001 Institute of Evidence-Based Chiropractic

Assessment:

Prognosis: Chiropractic care will continue to have a direct positive therapeutic effect on this significant neuromuscular condition, and it is my expectation that this patient will benefit from full recovery or at least improve function. The response to date is overall positive, with a percentage of improvement between 81%-99%. It is my clinical opinion that conservative chiropractic management should be continued. The patient tolerated today's treatment without incident.

Subluxation listing: C2L, C6R

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

Plan:

Chiropractic spinal manipulation was administered to reduce subluxations. Intersegmental mechanical traction was performed to mobilize the global restrictions in the spinal segments above and below the area of subluxation. An examination was performed today that was above and beyond the normal daily treatment evaluation. Manual manipulation performed at: C2, C6

No change in treatment plan or goals is needed at this time.

PLAN OF CARE:

Initial Date of Treatment: 1/14/21

Resolution of Chief Complaint: Pt has reached MMI

Progress Evaluation: Re-evaluation in approximately 120 days.

Frequency: Every two weeks until progress evaluation.

Prognosis: Good. It is my clinical opinion that conservative chiropractic management should be utilized for maintenance care.

Short term: Continue to stabilize the segments of the spine.

Long term: Prevent future dis-ease of the spine.



Electronically signed by: Heidi J. Pickering, DC

3/8/2022 11:26 AM

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DOCTOR/HOSPITAL LIEN

PATIENT IS RESPONSIBLE FOR ALL EXPENSES INCURRED REGARDLESS OF THE OUTCOME OF ANY SETTLEMENT, LITIGATION OR TRIAL

PATIENT: Jeff L. Plummer
HEALTH PROVIDER: Paradise Palms Chiropractic
19011 Dale Mabry Hwy.
Lutz, FL 33548
DATE OF ACCIDENT: January 05, 2021

PROTECTION OF OUTSTANDING CHARGES: If the above-named client recovers money damages from any person or entity responsible for charges incurred by the above-named health provider, we agree to withhold from any check or draft in which we are an additional named payee, sufficient funds, after deduction of attorney's fees and costs, to pay any outstanding medical bills in our possession and costs for any and all undisputed charges owed to you in connection with the accident or event giving rise to and covered by the recovery and not covered by any collateral source.

AMOUNT PROTECTED: The amount protected will only be the accident-related medical charges which are above and beyond those for which the insurance carrier(s) is/are legally responsible. For this letter to be valid, you will need to submit your bill to any applicable automobile and/or health insurance coverage which our client may have (e.g. PIP, group coverage, Medicare, etc.). It is the health provider's obligation to furnish us with periodic updates of outstanding charges. Otherwise, we will rely on previously received records in seeking reimbursement from the tortfeasor. Under no circumstances will we withhold a sum larger than that submitted to the tortfeasor for reimbursement.

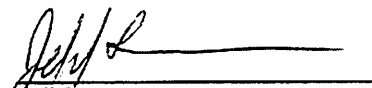
BALANCE CONFIRMATION: We will use best efforts to request a balance confirmation when recovery is imminent. If we fail to receive a written response within five (5) days of mailing, we will presume that the balance has been paid in full.

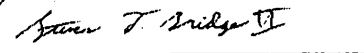
PRO RATA DISTRIBUTION IF INADEQUATE RECOVERY: If the net recovery is less than the total outstanding charges owed to all health providers covered by a Letter of Protection or any other lien holder, such funds will be distributed on a pro rata basis with the patient still responsible for the difference.

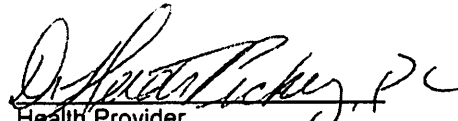
OUR RESPONSIBILITY ON FORENSIC SERVICES: This law firm acknowledges independent responsibility to the health providers for charges incurred for medical records and witness fees.

DISPUTES: If our client disputes any of your outstanding charges or claims a setoff and we are unable to resolve the issue, we will deposit the amount of the disputed charge/setoff into the Court Registry for judicial determination.

APPROVAL REQUIRED: This agreement becomes effective when all parties (client/patient, health provider and attorney) approve it in writing in the space provided below and return it to our office.


Jeff Plummer


Steven Bridge, Esq.


Health Provider

ORDER NOTE

Type: Radiology

Encounter Date: 2/2/2021

Paradise Palms Chiropractic

19011 N Dale Mabry

Lutz, FL 33548

(813) 948-1781

drheidi@drheidiparadise.com

Group NPI: 1356643423

Patient Name: Plummer, Jeffery

DOB: [REDACTED]

Account No: 1042

Rendering Physician: Heidi J. Pickering, DC

Individual NPI: 1356643423

Onset of current symptoms: 1/5/2021

Diagnosis:

S13.4XXA - Sprain of ligaments of cervical spine, initial encounter

S23.3XXA - Sprain of ligaments of thoracic spine, initial encounter

S33.5XXA - Sprain of ligaments of lumbar spine, initial encounter

M54.11 - Radiculopathy, occipito-atlanto-axial region

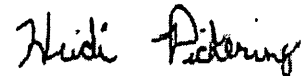
M54.13 - Radiculopathy, cervicothoracic region

M62.830 - Muscle spasm of back

Order:

Radiology:

Patient was referred for a lumbar spine MRI. Patient was referred for a cervical spine MRI.



Electronically signed by: Heidi J. Pickering, DC



2605 W. Kennedy Blvd
Tampa, FL 33609
Phone: (813) 876-7200
Fax: (813) 876-7300

Upright Open MRI, Xray & Ultrasound

Tampa • Brandon • Lakeland • Kissimmee • Orlando • Jacksonville • Orange Park • Palm Beach Gardens • Jupiter

PATIENT NAME:	JEFFREY PLUMMER	REFERRING PHYSICIAN:	HEIDI PICKERING
PATIENT ID:	682224	REFERRING PHONE:	
DOB:		REFERRING FAX:	
DOS:	02/10/2021		

EXAMINATION: MRI CERVICAL W/O

CLINICAL HISTORY: MVA (01/05/2020), neck pain.

TECHNIQUE: Multiplanar/multi-sequence MRI images through the cervical spine were obtained without intravenous gadolinium.

FINDINGS:

Cervical lordosis is maintained. Disc desiccation is seen at multiple levels. Grade-I retrolisthesis of C2 over C3 and C3 over C4 vertebrae is noted. C5-6 intervertebral disc space is reduced. Small spurs are seen at the anterior border of the cervical vertebrae. All the vertebrae in the view otherwise shows normal height, alignment and marrow signal intensities.

C2-3: No disc herniation / neuroforaminal compromise is seen.

C3-4: Broad based posterocentral disc herniation / protrusion is seen compressing the thecal sac.

C4-5: No disc herniation / neuroforaminal compromise is seen.

C5-6: No disc herniation / neuroforaminal compromise is seen.

C6-7: Broad based posterocentral disc herniation / protrusion is seen compressing the thecal sac and causing moderate narrowing of bilateral neural foramina. Mild central canal stenosis is seen with compression on the anterior border of the cord. Bilateral facet joint arthropathy is seen.

C7-T1: No disc herniation / neuroforaminal compromise is seen.

The cervical spinal cord shows normal signal intensity.

The atlanto-Odontoid space and atlantoaxial joint is unremarkable



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Upright Open MRI, Xray & Ultrasound

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PATIENT NAME:	JEFFREY PLUMMER	REFERRING PHYSICIAN:	HEIDI PICKERING
PATIENT ID:	682224	REFERRING PHONE:	
DOB:		REFERRING FAX:	
DOS:	02/10/2021		

EXAMINATION: MRI CERVICAL W/O

IMPRESSION:

1. Grade-I retrolisthesis of C2 over C3 and C3 over C4 vertebrae.
2. C3-4: Broad based posterocentral disc herniation / protrusion compressing the thecal sac.
3. C6-7: Broad based posterocentral disc herniation / protrusion compressing the thecal sac and causing moderate narrowing of bilateral neural foramina. Mild central canal stenosis is seen with compression on the anterior border of the cord. Bilateral facet joint arthropathy is seen.

Thank you, for this referral

Chintan Desai, MD
Board Certified, American Board of Radiology
ACR Certificate of Completion in NeuroRadiology
Fellowship Trained in Musculoskeletal MRI

This document has been electronically signed and approved

CHINTAN DESAI, MD
Electronically signed on: 2/11/2021 9:32:32 AM
Transcribed by: CD on: 2/11/2021 9:32:32 AM



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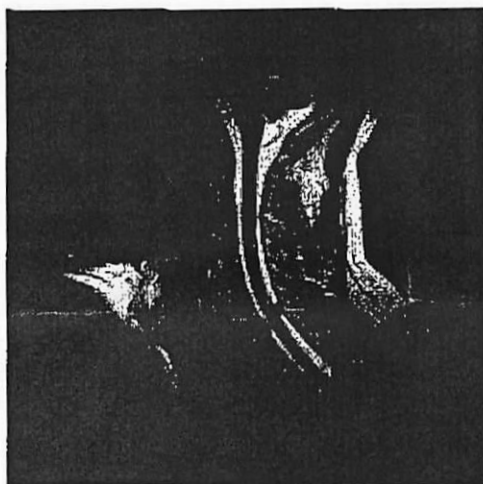
Upright Open MRI, Xray & Ultrasound

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PATIENT NAME: JEFFREY PLUMMER
PATIENT ID: 682224
DOB: XXXXXXXXXX
DOS: 02/10/2021

REFERRING PHYSICIAN: HEIDI PICKERING
REFERRING PHONE:
REFERRING FAX:

EXAMINATION: MRI CERVICAL W/O





2040 SHORT AVE, SUITE 100
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Phone: (727) 372-9922
Fax: (727) 372-8477

JEFFERY PLUMMER

Patient #: 37726

DOB: [REDACTED]

DOS: 03/17/2022

A summary of our findings are below beginning with the patient's history with regards to their spinal diagnosis, followed by our assessment and recommendations. Following that are our exam findings at the time of the visit. If you have any questions regarding your patient's care, please do not hesitate to call us at any time.

Attending Surgeon: VICTOR M HAYES MD

History of Present Illness (VICTOR M HAYES MD 03/17/2022 01:54 PM)

The patient is a 55 year old male who presents to discuss consultation. Follow up to discuss Orthopedic consult (The patient returns s/p an ACDF on 03/09/2022 and reports continued complaints of residual neck pain and shoulder pain but reports the numbness and paresthesias in his arms has improved but his headaches, neck pain, shoulder pain and tinnitus symptoms are slowly improving. he reports he is having difficulties swallowing. The patient denies use of the Hydrocodone and reports he has completed the antibiotics. The patient is continuing to wear the cervical collar as instructed. The incision site is clean and dry and free of infection. He reports his midback pain and low back symptoms are currently intermittent.).

Review of Systems (Autumn Altanburg; 03/17/2022 01:09 PM)

General: Present- **Fatigue.** Not Present- Fever, Night Sweats and Persistent Infections.

Skin: Not Present- Bruising.

HEENT: Present- **Headache** (occipital), **Blurred Vision** and **Tinnitus.**

Neck: Present- **Neck Pain** (pain radiates into bilateral periscapular shoulders and upper back (s/p ACDF at C6-7 on 03/09/2022)) and **Neck Stiffness.**

Respiratory: Not Present- Dyspnea.

Breast: Not Present- Breast Mass.

Cardiovascular: Not Present- Leg Cramps.

Gastrointestinal: Present- **Diarrhea.** Not Present- Heartburn.

Musculoskeletal: Present- **Back Pain** (upper back, midline at midback with anterior left rib pain and middle at low back), **Backache** and **Joint Pain.**

Neurological: Present- **Numbness** (bilateral arms (improved 3/17/22)), **Paresthesias** (bilateral arms (improved 3/17/22)) and **Weakness** (Worsening symptoms in that left arm). Not Present- Incontinence Stool, Incontinence Urine, Unsteadiness, Vertigo and Weakness In Extremities.

Psychiatric: Present- **Change In Sleep Pattern, Mood changes** and **Trouble Falling Asleep.** Not Present- Anxiety.

Endocrine: Not Present- Appetite Changes.

Hematology: Not Present- Abnormal Bleeding.

JEFFERY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Allergies (Autumn Altenburg; 03/17/2022 01:10 PM)
No Known Drug Allergies 03/04/2021

Problem List/Past Medical History (Autumn Altenburg; 03/17/2022 01:10 PM)
LUMBAGO SYNDROME (724.2 | M54.5)
MUSCLE SPASM (728.85 | M62.838)
PAIN IN THORACIC SPINE (724.1 | M54.6)
DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (722.10 | M51.26)
PRE-OP CHEST EXAM (V72.82 | Z01.811)
CORD COMPRESSION (336.9 | G95.20)
No pertinent Spine Related past medical history
CERVICAL DISC DISORDER (722.91 | M50.90)
CERVICALGIA (723.1 | M54.2)

Family History (Autumn Altenburg; 03/17/2022 01:10 PM)
Family History Reviewed - no changes From Previous Visit

Social History (Autumn Altenburg; 03/17/2022 01:10 PM)
Marital status: Single
Current tobacco use: Never smoker
No Drug Use
Non Drinker/No Alcohol Use
Current Household Members: Friends
Current work status: Self-employed; automomotive industry

Medication History (Autumn Altenburg; 03/17/2022 01:10 PM)
Cefdinir (300MG Capsule 1 (one) Capsule Oral two times daily, Taken starting 03/07/2022) Active - Hx Entry. (Complete all meds as prescribed.)
Doxycycline Hyclate (100MG Tablet 1 (one) Tablet Oral two times daily, Taken starting 03/07/2022) Active - Hx Entry. (Complete all meds as prescribed.)
Medications Reconciled.

Past Surgical (Autumn Altenburg; 03/17/2022 01:10 PM)
ACDF @ C6-7 (03/09/2022); with Dr. Victor M Hayes, M.D

Diagnostic Studies (Autumn Altenburg; 03/17/2022 01:10 PM)
MRI, Lumbar Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 anterolisthesis of L5 over S1. Grade 1 retrolisthesis of L1 over L2 and L2 over L3. Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5.
MRI, Cervical Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 retrolisthesis of C2 over C3 and C3 over C4. Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord.
Cervical Spine X-ray (03/17/2022); In Office: Instrumentation in good position.

Vitals (Autumn Altenburg; 03/17/2022 01:11 PM)
03/17/2022 01:10 PM
Weight: 201 lb **Height:** 72 in
Body Surface Area: 2.15 m² **Body Mass Index:** 27.26 kg/m²
Pain level: 3/10
Pulse: 78 (Regular) **Resp.:** 16 (Unlabored) **P. OX:** 99% (Room air)
BP: 133/86 Manual (Sitting, Left Arm, Standard)

Physical Exam (VICTOR M HAYES MD; 03/17/2022 01:55 PM)
The physical exam findings are as follows:

General
General Appearance - Cooperative. **Posture** - Normal posture.

Integumentary
Global Assessment: Upon inspection and palpation of skin surfaces of the - Normal skin inspection of Neck, Trunk, RUE, RLE, LUE, LLE unless detailed below.

JEFFERY PLUMMER

Patient #: 37726

DOB:

Peripheral Vascular

Upper Extremity: Inspection - Bilateral - Normal - No Clubbing, No Cyanosis, No Edema, Pulses Intact unless detailed below.

Lower Extremity: Inspection - Bilateral - Normal - No Clubbing, No Cyanosis, No Edema, Pulses Intact unless detailed below.

Neurologic

Sensory: Paresthesia - no paresthesia, normal pressure sensation and no numbness.

Overall Assessment of Muscle Strength and Tone reveals: Muscle Tone - neck: normal, lumbar: spasm, LUE: normal, RUE: normal, LLE: normal and RLE: normal.

General Assessment of Reflexes: Right Wrist - 1+. Types - Bilateral - all DTR 2+ unless otherwise noted.

Pathological Reflexes (Hoffman, Inverted Radial) - Bilateral - None. Plantar Reflexes (L4-S2) - Bilateral - Absent. Coordination - Normal.

Neuropsychiatric

Mental status exam performed with findings of - Oriented X3 with appropriate mood and affect.

Musculoskeletal

Global Assessment

All Normal - Spine, BUE, BLE, Inspection, Palpation, ROM, Stability, Tone: Normal unless noted (Motor in neuro exam).

Coordination - finger-nose, Romberg, dysmetria, uln. drift: WNL. Gait and Station - difficulty with tandem walk (mild balance issues), toe walking: able, heel walking: able, squatting: able and single stance: able. Assistive Devices - none. Special tests - forward flexion to knees, spurlings test negative, str. leg. raise: bilateral lower back pain, contralateral SLR: bilateral negative and femoral stretch: bilateral negative. Inspection and Palpation - head: normal inspection, LUE: normal inspection, RUE: normal inspection, LLE: normal inspection and RLE: normal inspection. Assessment of Spine/Joint Stability - head: normal (no instability), neck: normal (no instability), thoracic: normal (no instability), lumb: normal (no instability), LUE: normal (no instability), RUE: normal (no instability), LLE: normal (no instability) and RLE: normal (no instability). ROM - neck: painful, neck: stiffness, lumb: painful, lumb: stiffness, LUE: normal, RUE: normal, LLE: normal and RLE: normal.

Spine/Ribs/Pelvis

Cervical Spine:

Inspection and Palpation: Tenderness - mild and moderate. Swelling - none. Wound - Clean, Dry, Intact.

Thoracic (Dorsal) Spine:

Inspection and Palpation: Tenderness - None. Swelling - none.

Lumbosacral Spine:

Inspection and Palpation: Tenderness - moderate. Swelling - none.

Upper Extremity

Shoulder: Impingement - impingement test negative, (R) and impingement test negative, (L).

Elbow: Functional Testing - Tinel's Sign negative, (R) and Tinel's Sign negative, (L).

Hand/Wrist:

Wrist: Functional Testing - Phalen's Sign negative, (R), Phalen's Sign (Prayer Sign) negative, (L), Tinel's Sign negative, (R) and Tinel's Sign negative, (L).

Lower Extremity

Hip: ROM - NO Left Hip Pain with ROM and NO Right Hip Pain with ROM.

Knee/Patella: ROM - NO Left Knee Pain with ROM and NO Right Knee Pain with ROM.

Lymphatic

General Lymphatics

All Normal - Head, Neck, Axillary, Inguinal Lymphatics Normal unless otherwise noted..

Assessment & Plan (VICTOR M HAYES MD; 03/17/2022 01:56 PM)

CERVICALGIA (723.1 | M54.2)

Current Plans:

- X-RAY EXAM OF NECK SPINE (72040) Routine

CORD COMPRESSION (336.9 | G95.20)

Current Plans:

CERVICAL DISC DISORDER (722.91 | M50.90)

Current Plans:

MUSCLE SPASM (728.85 | M62.838)

Current Plans:

DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (722.10 | M51.26)

Current Plans:

PAIN IN THORACIC SPINE (724.1 | M54.6)

Current Plans:

LUMBAGO SYNDROME (724.2 | M54.5)

Current Plans:

- Cervical recommendations: Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord; Neck pain radiculopathy; Status post ACDF doing well
 1. Taper brace and increase activity level
 - 2 per it adjacent level protocolLumbar Recommendations: Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5; lower back pain
 1. Cautious use of NSAIDs.
 2. continued therapy
 3. Epidural injection at L4-5 with facet blocks series(HOLD)
 4. If pain is not improved can consider transforaminal discectomy at L4-5(MENDS endoscopic assistance) L4 and L5 and S1 medial lateral branch neurotomy as alternative to fusion(HOLD)Follow-up: S/p acdf f/u in 6 weeks

Victor m 1 day mo

VICTOR M HAYES MD

JEFFERY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Elite Spine and Orthopedics



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JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

DOS: 07/08/2024

A summary of our findings are below beginning with the patient's history with regards to their spinal diagnosis, followed by our assessment and recommendations. Following that are our exam findings at the time of the visit. If you have any questions regarding your patient's care, please do not hesitate to call us at any time.

Attending Surgeon: VICTOR M. HAYES MD

History of Present Illness (VICTOR M. HAYES MD 07/08/2024 09:29 AM)

The patient is a 57 year old male who presents to discuss consultation. Follow up to discuss Orthopedic consult (The patient returns s/p an ACDF on 03/09/2022 and reports continued complaints of residual Posterior neck pain that radiates into bilateral shoulders and bilateral arms to the hand associated with numbness and paresthesias in bilateral arms and hands intermittently. He reports his symptoms have been constant since his last office visit in 2022 but the he does feel his symptoms have improved and are better than they were before surgery. The patient reports his low back symptoms have been improving since his last visit as well. He states he takes Advil as needed for the pain.). He has residual tingling in that left arm which was the worst arm pre-op: HE will see Dr Donovan for both hands Left > Right. the patient is a mechanic and when he works and looks up he has worsening numbness and neck pain and issue with him. When he is working 7 more difficulty with this.

Allergies (Kacilee Cummings; 07/08/2024 08:49 AM)**No Known Drug Allergies** 03/04/2021**Past Medical History** (Kacilee Cummings; 07/08/2024 08:48 AM)**No pertinent Spine Related past medical history****CERVICAL DISC DISORDER (M50.90)****MUSCLE SPASM (M62.838)****CORD COMPRESSION (G95.20)****CERVICALGIA (M54.2)****PRE-OP CHEST EXAM (Z01.811)****FOLLOW-UP EXAMINATION AFTER ORTHOPEDIC SURGERY (Z09)****ARTHRODESIS STATUS (Z98.1)****LUMBAGO SYNDROME (M54.5)****PAIN IN THORACIC SPINE (M54.6)****DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (M51.26)****Family History** (Kacilee Cummings; 07/08/2024 08:49 AM)**No pertinent family history: With regards to Spinal Disorder****Family History Reviewed - no changes From Previous Visit**

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Monday, July 8, 2024

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000214

Social History (Kacilee Cummings; 07/08/2024 08:50 AM)**Marital status:** Divorced**Current tobacco use:** Never smoker**No Drug Use****Non Drinker/No Alcohol Use****Current Household Members:** Alone**Current work status:** Self-employed; Internet Sales, Carparts & Mechanic**Medication History** (Kacilee Cummings; 07/08/2024 08:51 AM)

Advil (1 (one) Oral as needed) Specific strength unknown - Active.

Past Surgical (Kacilee Cummings; 07/08/2024 08:51 AM)**ACDF @ C6-7** (03/09/2022); with Dr. Victor M Hayes, M.D**Diagnostic Studies** (VICTOR M. HAYES MD; 07/08/2024 09:28 AM)**MRI, Lumbar Spine** (02/10/2021); ADG: PACS and reports reviewed: Grade 1 anterolisthesis of L5 over S1. Grade 1 retrolisthesis of L1 over L2 and L2 over L3. Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5.**MRI, Cervical Spine** (02/10/2021); ADG: PACS and reports reviewed: Grade 1 retrolisthesis of C2 over C3 and C3 over C4. Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord.**Cervical Spine X-ray** (07/08/2024); In Office 2 view AP/ Lat (VMH): Instrumentation in good position. FUSED Well

03/17/2022: In Office 2 view AP/ Lat (VMH): Instrumentation in good position.

Review of Systems (Autumn Altenburg; 07/08/2024 09:01 AM)**General Present- Fatigue and Night Sweats.** Not Present- Fever and Persistent Infections.**Skin** Not Present- Bruising.**HEENT** Not Present- Headache.**Neck Present- Neck Pain** (pain radiates into bilateral periscapular shoulders and upper back (s/p ACDF at C6-7 on 03/09/2022)) and **Neck Stiffness.****Respiratory** Not Present- Dyspnea.**Breast** Not Present- Breast Mass.**Cardiovascular** Not Present- Leg Cramps.**Gastrointestinal** Not Present- Heartburn.**Musculoskeletal Present- Back Pain** (upper back, midline at midback with anterior left rib pain and middle at low back), **Backache** and **Joint Pain.****Neurological Present- Numbness** (bilateral arms and hands), **Paresthesias** (bilateral arms and hands) and **Trouble walking** (antalgic gait and balance issues). Not Present- Incontinence Stool, Incontinence Urine, Unsteadiness, Vertigo and Weakness In Extremities.**Psychiatric Present- Change in Sleep Pattern, Mood changes and Trouble Falling Asleep.** Not Present- Anxiety.**Endocrine** Not Present- Appetite Changes.**Hematology** Not Present- Abnormal Bleeding.**Vitals** (Autumn Altenburg; 07/08/2024 09:13 AM)

07/08/2024 09:04 AM

Weight: 198 lb **Height:** 72 in**Body Surface Area:** 2.12 m² **Body Mass Index:** 26.85 kg/m²**Pain level:** 4/10**Pulse:** 64 (Regular) **Resp.:** 16 (Unlabored) **P. OX:** 99% (Room air)**BP:** 129/74 Manual (Sitting, Left Arm, Standard)

Physical Exam (Autumn Altenburg; 07/08/2024 09:21 AM)

The physical exam findings are as follows:

General

General Appearance - Cooperative.

Posture - Normal posture.

Integumentary**Global Assessment**

Upon inspection and palpation of skin surfaces of the - Normal skin inspection of Neck, Trunk, RUE, RLE, LUE, LLE unless detailed below.

Peripheral Vascular**Upper Extremity**

Inspection - Bilateral - Normal - No Clubbing, No Cyanosis, No Edema, Pulses Intact unless detailed below.

Lower Extremity

Inspection - Bilateral - Normal - No Clubbing, No Cyanosis, No Edema, Pulses Intact unless detailed below.

Neurologic**Sensory**

Paresthesia - no paresthesia, normal pressure sensation and **numbness present** (numbness left arm) .

Overall Assessment of Muscle Strength and Tone reveals

Muscle Tone - neck: normal, **lumbar: spasm**, LUE: normal, RUE: normal, LLE: normal and RLE: normal.

General Assessment of Reflexes

Right Wrist - 1+. **Types - Bilateral** - all DTR 2+ unless otherwise noted - Bilateral.

Pathological Reflexes (Hoffman, Inverted Radial) - Bilateral - None.

Plantar Reflexes (L4-S2) - Bilateral - Absent - Bilateral.

Coordination - Normal.

Neuropsychiatric

Mental status exam performed with findings of - Oriented X3 with appropriate mood and affect.

Musculoskeletal**Global Assessment**

All Normal - Spine, BUE, BLE, Inspection, Palpation, ROM, Stability, Tone: Normal unless noted (Motor in neuro exam). **Coordination** - finger-nose, Romberg, dysmetria, uln. drift: WNL. **Gait and Station** - **difficulty with tandem walk (mild balance issues)**, toe walking: able, heel walking: able, squatting: able and single stance: able. **Assistive Devices** - none. **Special tests** - **forward flexion to knees**, spurlings test negative, **str. leg. raise: bilateral lower back pain**, contralateral SLR: bilateral negative and femoral stretch: bilateral negative. **Inspection and Palpation** - head: normal inspection, LUE: normal inspection, RUE: normal inspection, LLE: normal inspection and RLE: normal inspection. **Assessment of Spine/Joint Stability** - head: normal (no instability), neck: normal (no instability), thoracic: normal (no instability), lumb: normal (no instability), LUE: normal (no instability), RUE: normal (no instability), LLE: normal (no instability) and RLE: normal (no instability). **ROM** - **neck: painful, neck: stiffness**, **lumb: painful, lumb: stiffness**, LUE: normal, RUE: normal, LLE: normal and RLE: normal.

Spine/Ribs/Pelvis

Cervical Spine: Inspection and Palpation - Tenderness - mild and moderate. **Swelling** - none. **Wound** - Clean, Dry, Intact.

Thoracic (Dorsal) Spine: Inspection and Palpation - Tenderness - None. **Swelling** - none.

Lumbosacral Spine: Inspection and Palpation - Tenderness - moderate. **Swelling** - none.

Upper Extremity

Shoulder - Impingement - impingement test negative, (R), impingement test negative, (L). **Elbow - Functional Testing** - Tinel's Sign negative, (R), Tinel's Sign negative, (L).

Hand/Wrist: Wrist - Functional Testing - Phalen's Sign negative, (R), Phalen's Sign (Prayer Sign) negative, (L), Tinel's Sign negative, (R), Tinel's Sign negative, (L).

Hand:

Assessment of pain reveals the following findings: - **Location** - **palmar surface, (R)** (trigger fingers pain Left >>> Right) and **palmar surface, (L)** (+++ Dupetrens) .

Lower Extremity

Hip - ROM - NO Left Hip Pain with ROM, NO Right Hip Pain with ROM. **Knee/Patella - ROM** - NO Left Knee Pain with ROM, NO Right Knee Pain with ROM.

Lymphatic**General Lymphatics**

All Normal - Head, Neck, Axillary, Inguinal Lymphatics Normal unless otherwise noted..

Assessment & Plan (VICTOR M. HAYES MD; 07/08/2024 09:29 AM)

MUSCLE SPASM (M62.838)

Current Plans:

CORD COMPRESSION (G95.20)

Current Plans:

- MRI OF CERVICAL SPINE W/O CONTRAST (72141) Routine ()

CERVICALGIA (M54.2)

Current Plans:

- X-RAY EXAM OF NECK SPINE (72040) Routine ()

DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (M51.26)

Current Plans:

PAIN IN THORACIC SPINE (M54.6)

Current Plans:

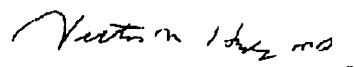
LUMBAGO SYNDROME (M54.5)

Current Plans:

- Cervical recommendations: Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord: Neck pain radiculopathy: Status post ACDF FUSED: Residual left arm numbness:
 1. Adjacent level protocol
 2. Bilateral trigger finger : Se Dr Donovan:
 3. MRI cervical for persistent left arm numbness: ALD vs. chronic nerve damage ..But I want him to see Dr Donovan first.Trigger Fingers Left > Right Recommendations
 1. Dr Donovan for trigger injections

Lumbar Recommendations: Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5; lower back pain

1. Cautious use of NSAIDs.
 2. Continued therapy
 3. Epidural injection at L4-5 with facet blocks series option
 4. If pain is not improved can consider transforaminal discectomy at L4-5 (MENDS endoscopic assistance) L4 and L5 and S1 medial lateral branch neurectomy as alternative to fusion (HOLD)
- Follow-up: s/p ACDF with bilateral left > right trigger fingers



VICTOR M. HAYES MD

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ODESSA, FL 33556
Phone: (727) 372-9922
Fax: (727) 372-8477

JEFFREY PLUMMER
Patient #: 37726
DOB: [REDACTED]

DOS: 07/17/2024

A summary of our findings are below beginning with the patient's history with regards to their spinal diagnosis, followed by our assessment and recommendations. Following that are our exam findings at the time of the visit. If you have any questions regarding your patient's care, please do not hesitate to call us at any time.

Attending Surgeon: VICTOR M. HAYES MD

History of Present Illness (JAMES DONOVAN 07/23/2024 08:24 AM)
Patient words: 58-year-old male comes in for evaluation of hand pain. Patient has been noticing swelling and pain with numbness and tingling in both hands since his motor vehicle accident 1/5/2021. Spent having a lot of spine issues and been dealing with different physicians related to that. Has also been noticing that the hands were numb. He has also noticed that this is associated with swelling of the hands. Sometimes having locking of the fingers.

The patient is a 57 year old male who presents with a complaint of Accident. The date of the accident was 01/05/2021. The accident was a MVA (driver, SUV was rear ended (Hard hit: hitch receiver strike). The patient reports he felt his symptoms shortly following the accident. Since the accident the patient has had neck pain (associated with occipital region headaches and tinnitus and blurred vision), arm numbness and paresthesias (bilateral), shoulder pain (bilateral periscapular) and back pain (upper back and midline at midback with left anterior rib pain and middle at low back).

Prior medical treatments for this accident include the following: chiropractic adjustments, home exercise program and Chiropractic treatment (with Paradise Palms Chiropractic 3x per week for 1 month and continuing (15 or so). The patients symptoms have been increasing despite the treatment detailed in this history.

The patients pre-accident history is detailed below. The patient does not recall significant neck pain, arm pain, back pain and leg pain immediately prior to the accident. The patient's baseline was that they do not recall receiving medication and/or significant treatment for neck pain, arm pain, back pain and leg pain immediately prior to accident. The patient states that they had a prior history of back pain (low back prior history of chiropractic treatment approximately 30 years ago). The patient reports his low back pain had improved and been asymptomatic prior to the MVA on 01/05/2021; however, since the MVA on 01/05/2021 his low back pain has been severe. The patient reports having been involved in 1 accident but does not recall having sustained injuries and/or received treatment prior to the MVA on 01/05/2021. The patient does not recall having seen an Orthopedic Spine Surgeon but reports he may have completed x-rays of the spine but does not recall having completed MRI/CT scan of the spine prior to the MVA on 01/05/2021.

Allergies (Destiny Ortega; 07/17/2024 10:16 AM)
No Known Drug Allergies 03/04/2021

Past Medical History (Destiny Ortega; 07/17/2024 10:16 AM)
No pertinent Spine Related past medical history
ARTHRODESIS STATUS (Z98.1)
MUSCLE SPASM (M62.838)
FOLLOW-UP EXAMINATION AFTER ORTHOPEDIC SURGERY (Z09)
PRE-OP CHEST EXAM (Z01.811)
CERVICAL DISC DISORDER (M50.90)
CERVICALGIA (M54.2)
LUMBAGO SYNDROME (M54.5)
PAIN IN THORACIC SPINE (M54.6)
DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (M51.26)
CORD COMPRESSION (G95.20)

Family History (Destiny Ortega; 07/17/2024 10:16 AM)
No pertinent family history: With regards to Spinal Disorder
Family History Reviewed - no changes From Previous Visit

Social History (Destiny Ortega; 07/17/2024 10:16 AM)
Marital status: Divorced
Current tobacco use: Never smoker
No Drug Use
Non Drinker/No Alcohol Use
Current Household Members; Alone
Current work status: Self-employed; Internet Sales, Carparts & Mechanic

Medication History (Destiny Ortega; 07/17/2024 10:16 AM)
Advil (1 (one) Oral as needed) Specific strength unknown - Active.
Medications Reconciled.

Past Surgical (Destiny Ortega; 07/17/2024 10:16 AM)
ACDF @ C6-7 (03/09/2022); with Dr. Victor M Hayes, M.D

Diagnostic Studies (Destiny Ortega; 07/17/2024 10:16 AM)
MRI, Lumbar Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 anterolisthesis of L5 over S1. Grade 1 retrolisthesis of L1 over L2 and L2 over L3. Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5.
MRI, Cervical Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 retrolisthesis of C2 over C3 and C3 over C4. Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord.
Cervical Spine X-ray (07/08/2024); In Office 2 view AP/ Lat (VMH): Instrumentation in good position. FUSED Well

03/17/2022:In Office 2 view AP/ Lat (VMH): Instrumentation in good position.

Review of Systems (Destiny Ortega; 07/17/2024 10:16 AM)

General Present- Fatigue and Night Sweats. Not Present- Fever and Persistent Infections.
Skin Not Present- Bruising.
HEENT Not Present- Headache.
Neck Present- Neck Pain (pain radiates into bilateral periscapular shoulders and upper back (s/p ACDF at C6-7 on 03/09/2022)) and Neck Stiffness.
Respiratory Not Present- Dyspnea.
Breast Not Present- Breast Mass.
Cardiovascular Not Present- Leg Cramps.
Gastrointestinal Not Present- Heartburn.
Musculoskeletal Present- Back Pain (upper back, midline at midback with anterior left rib pain and middle at low back), **Backache** and Joint Pain.
Neurological Present- Numbness (bilateral arms and hands), **Paresthesias** (bilateral arms and hands) and **Trouble walking** (antalgic gait and balance issues). Not Present- Incontinence Stool, Incontinence Urine, Unsteadiness, Vertigo and Weakness In Extremities.
Psychiatric Present- Change in Sleep Pattern, Mood changes and Trouble Falling Asleep. Not Present- Anxiety.
Endocrine Not Present- Appetite Changes.
Hematology Not Present- Abnormal Bleeding.

Vitals (Destiny Ortega; 07/17/2024 10:17 AM)

07/17/2024 10:16 AM
Weight: 198 lb, 8 oz **Height:** 72 in
Body Surface Area: 2.12 m² **Body Mass Index:** 26.92 kg/m²
Pain level: 2/10
Resp.: 17 (Unlabored) **P. OX:** 98% (Room air)

Physical Exam (JAMES DONOVAN; 07/23/2024 08:29 AM)
The physical exam findings are as follows:

General
Mental Status - Alert.
General Appearance - Cooperative, Well groomed, Not in acute distress.
Orientation - Oriented X3.
Build & Nutrition - Well nourished.

Musculoskeletal
Upper Extremity
Elbow - Functional Testing - Pronator Teres Syndrome Test negative, (R), Pronator Teres Syndrome Test negative, (L), Tinel's Sign negative, (R), Tinel's Sign negative, (L).
Hand/Wrist: Wrist - Functional Testing - Tinel's Sign positive, (R) and Tinel's Sign positive, (L). **Hand - Evaluation of related systems reveals** - no digital clubbing or cyanosis. **Examination of the right hand reveals** - no tenderness to palpation, no pain, normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities. **Examination of the left hand reveals** - no tenderness to palpation, no pain, normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities.
Strength and Tone: Right - Grip Strength - 100 LBS. **Pulp to Pulp Pinch Strength** - 19 LBS. **Left - Grip Strength** - 90 LBS. **Pulp to Pulp Pinch Strength** - 13 LBS. **Hand - Deformities/Malalignments/Discrepancies** - Dupuytren's contracture, (R) (cord formation) and Dupuytren's contracture, (L) (cord formation) .

Assessment & Plan (JAMES DONOVAN; 07/23/2024 08:31 AM)

MOTOR VEHICLE ACCIDENT (V89.2XXA)
Current Plans:

CERVICALGIA (M54.2)
Current Plans:

PAIN OF LEFT HAND (M79.642)

Current Plans:

- Cervical recommendations: Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord: Neck pain radiculopathy: Status post ACDF FUSED: Residual left arm numbness:
 1. Adjacent level protocol
 2. Bilateral trigger finger : Se Dr Donovan:
 3. MRI cervical for persistent left arm numbness: ALD vs. chronic nerve damage ..But I want him to see Dr Donovan first.

Lumbar Recommendations: Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5; lower back pain

1. Cautious use of NSAIDs.
2. Continued therapy
3. Epidural injection at L4-5 with facet blocks series option
4. If pain is not improved can consider transforaminal discectomy at L4-5(MENDS endoscopic assistance) L4 and L5 and S1 medial lateral branch neurectomy as alternative to fusion(HOLD)

Bilateral Hand Recommendations: Dupuytren's disease of the palm with cord, numbness tingling/carpal tunnel, trigger digits

1. 7/17/24-right and left carpal tunnel injections given
2. Fit bracing perfect hyperflexion of wrist while sleeping at, the cuff avoidance of aggravating activities and ergonomic changes
3. Discussed Dupuytren's disease of the palm and treatment options if symptoms persist

Follow-up: 8 weeks repeat evaluation hand

NUMBNESS AND TINGLING (R20.0)Current Plans:**CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)**Current Plans:

- CARPAL TUNNEL INJECTION (20526) Routine (Verbal consent was obtained. Betadine was used to cleanse the area. Ethyl chloride was sprayed to numb the area. 1 cc of lidocaine plain with 0.5 cc of methylprednisolone at 40 mg per cc was given. Band-aid was applied.)
- INJECTION OF METHYLPREDNISOLONE ACETATE, NOW (96372) Routine ()
- Patient Education: Diabetes Overview (Living with Diabetes): blood sugar

PAIN OF RIGHT HAND (M79.641)Current Plans:**CARPAL TUNNEL SYNDROME OF RIGHT WRIST (G56.01)**Current Plans:

- INJECTION OF METHYLPREDNISOLONE ACETATE, NOW (96372) Routine ()
- CARPAL TUNNEL INJECTION (20526) Routine (Verbal consent was obtained. Betadine was used to cleanse the area. Ethyl chloride was sprayed to numb the area. 1 cc of lidocaine plain with 0.5 cc of methylprednisolone at 40 mg per cc was given. Band-aid was applied.)
- Patient Education: Carpal Tunnel Syndrome: hand pain
- Home exercise program for the hand/fingers
- How to access health information online

EXERCISE COUNSELING (Z71.82)

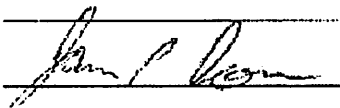
Current Plans:

- Education and Counseling
- Education and Counseling

PALMAR FASCIAL FIBROMATOSIS [DUPUYTREN] (M72.0)

Current Plans:

- Patient Education: Dupuytren's Contracture: hand
- Surgical Discussion



JAMES DONOVAN

Elite Spine and Orthopedics



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JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

DOS: 09/17/2024

A summary of our findings are below beginning with the patient's history with regards to their spinal diagnosis, followed by our assessment and recommendations. Following that are our exam findings at the time of the visit. If you have any questions regarding your patient's care, please do not hesitate to call us at any time.

Attending Surgeon: VICTOR M. HAYES MD

History of Present Illness (JAMES DONOVAN 09/19/2024 09:51 AM)

Patient words: Patient presents for follow-up evaluation of bilateral hand pain. Patient has been noticing swelling and pain with numbness and tingling in both hands since his motor vehicle accident 1/5/2021. Previous discussion about carpal tunnel syndrome and Dupuytren's contracture. Previous carpal tunnel injections provided no relief. Patient is using wrist braces at night. States oftentimes symptoms are worse at night.

The patient is a 58 year old male who presents to discuss consultation. Follow up to discuss Orthopedic consult (The patient comes in today with complaints of bilateral hand pain. He is experiencing pain with numbness and tingling in both hands associated with stiffness and tightness in fingers on both hands possibly trigger finger. The patient had injections on 7/17/24 in both hands and did not get any relief from the injections. He is currently wearing his hand braces pm. He does take Advil prn for pain. He also reports secondary complaints of bilateral neck pain associated with occipital headaches and numbness and tingling shooting down his left and right arms to the elbows and intermittently to the fingertips as well as mid back pain with left rib pain and lower back pain in the middle without radiating symptoms but does report numbness and tingling in the left foot.).

Allergies (Kaciles Cummings; 09/17/2024 11:58 AM)**No Known Drug Allergies** 03/04/2021**Past Medical History** (MICHAEL GOETZ PA-C; 09/17/2024 12:47 PM)**No pertinent Spine Related past medical history****LUMBAGO SYNDROME (M54.5)****CERVICAL DISC DISORDER (M50.90)****PAIN IN THORACIC SPINE (M54.6)****DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (M51.26)****ARTHRODESIS STATUS (Z98.1)****MUSCLE SPASM (M62.838)****FOLLOW-UP EXAMINATION AFTER ORTHOPEDIC SURGERY (Z09)****PRE-OP CHEST EXAM (Z01.811)****CORD COMPRESSION (G95.20)****CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)****PAIN OF RIGHT HAND (M79.641)****CARPAL TUNNEL SYNDROME OF RIGHT WRIST (G56.01)****PALMAR FASCIAL FIBROMATOSIS [DUPUYTREN] (M72.0)****MOTOR VEHICLE ACCIDENT (V89.2XXA)****CERVICALGIA (M54.2)****PAIN OF LEFT HAND (M79.642)**

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Thursday, September 19, 2024

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NUMBNESS AND TINGLING (R20.0)**Family History** (Kacilee Cummings; 09/17/2024 11:58 AM)**No pertinent family history: With regards to Spinal Disorder****Family History Reviewed - no changes From Previous Visit****Social History** (Kacilee Cummings; 09/17/2024 11:58 AM)**Marital status:** Divorced**Current tobacco use:** Never smoker**No Drug Use****Non Drinker/No Alcohol Use****Current Household Members;** Alone**Current work status:** Self-employed; Internet Sales, Carparts & Mechanic**Medication History** (MICHAEL GOETZ PA-C; 09/17/2024 12:47 PM)

Advil (1 (one) Oral as needed) Specific strength unknown - Active.

Medications Reconciled.

Other Problems (MICHAEL GOETZ PA-C; 09/17/2024 12:47 PM)**EXERCISE COUNSELING (Z71.82)****Past Surgical** (Kacilee Cummings; 09/17/2024 11:59 AM)**ACDF @ C6-7** (03/09/2022); with Dr. Victor M Hayes, M.D**Diagnostic Studies** (Kacilee Cummings; 09/17/2024 11:59 AM)**MRI, Lumbar Spine** (02/10/2021); ADG: PACS and reports reviewed: Grade 1 anterolisthesis of L5 over S1. Grade 1 retrolisthesis of L1 over L2 and L2 over L3. Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5.**MRI, Cervical Spine** (02/10/2021); ADG: PACS and reports reviewed: Grade 1 retrolisthesis of C2 over C3 and C3 over C4. Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord.**Cervical Spine X-ray** (07/08/2024); In Office 2 view AP/ Lat (VMH): Instrumentation in good position. FUSED Well

03/17/2022: In Office 2 view AP/ Lat (VMH): Instrumentation in good position.

Review of Systems (Kacilee Cummings; 09/17/2024 11:57 AM)**General Present-** Fatigue and **Night Sweats**. Not Present- Fever and Persistent Infections.**Skin** Not Present- Bruising.**HEENT** Not Present- Headache.**Neck Present-** Neck Pain (pain radiates into bilateral periscapular shoulders and upper back (s/p ACDF at C6-7 on 03/09/2022)) and Neck Stiffness.**Respiratory** Not Present- Dyspnea.**Breast** Not Present- Breast Mass.**Cardiovascular** Not Present- Leg Cramps.**Gastrointestinal** Not Present- Heartburn.**Musculoskeletal Present-** Back Pain (upper back, midline at midback with anterior left rib pain and middle at low back), Backache and Joint Pain (bilateral hands).**Neurological Present-** Numbness (bilateral arms and hands), Paresthesias (bilateral arms and hands) and Trouble walking (antalgic gait and balance issues). Not Present- Incontinence Stool, Incontinence Urine, Unsteadiness, Vertigo and Weakness In Extremities.**Psychiatric Present-** Change in Sleep Pattern, Mood changes and Trouble Falling Asleep. Not Present- Anxiety.**Endocrine** Not Present- Appetite Changes.**Hematology** Not Present- Abnormal Bleeding.**Vitals** (Kacilee Cummings; 09/17/2024 12:00 PM)

09/17/2024 11:59 AM**Weight:** 206 lb **Height:** 72 in**Body Surface Area:** 2.16 m² **Body Mass Index:** 27.94 kg/m²**Pain level:** 1/10**Pulse:** 98 (Regular) **Resp.:** 17 (Unlabored) **P. OX:** 99% (Room air)**BP:** 143/91 Manual (Sitting, Left Arm, Standard)**Physical Exam** (JAMES DONOVAN: 09/19/2024 09:51 AM)

The physical exam findings are as follows:

General**Mental Status** - Alert.**General Appearance** - Cooperative, Well groomed, Not in acute distress.**Orientation** - Oriented X3.**Build & Nutrition** - Well nourished.**Musculoskeletal****Upper Extremity****Elbow - Functional Testing** - Pronator Teres Syndrome Test negative, (R), Pronator Teres Syndrome Test negative, (L), Tinel's Sign negative, (R), Tinel's Sign negative, (L).**Hand/Wrist: Wrist - Functional Testing** - Tinel's Sign negative, (R), Tinel's Sign negative, (L). **Hand - Evaluation of related systems reveals** - no digital clubbing or cyanosis. **Examination of the right hand reveals** - no tenderness to palpation, no pain, normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities. **Examination of the left hand reveals** - no tenderness to palpation, no pain, normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities.**Strength and Tone: Right - Grip Strength** - 70 LBS. **Pulp to Pulp Pinch Strength** - 19 LBS. **Left - Grip Strength** - 100 LBS. **Pulp to Pulp Pinch Strength** - 19 LBS. **Hand - Deformities/Malalignments/Discrepancies** - Dupuytren's contracture, (R) (cord formation) and Dupuytren's contracture, (L) (cord formation) .**Assessment & Plan** (JAMES DONOVAN: 09/19/2024 09:52 AM)**MOTOR VEHICLE ACCIDENT (V89.2XXA)**Current Plans:**PALMAR FASCIAL FIBROMATOSIS [DUPUYTREN] (M72.0)**Current Plans:**CARPAL TUNNEL SYNDROME OF RIGHT WRIST (G56.01)**Current Plans:

- CARPAL TUNNEL INJECTION (20526) Routine (Verbal consent was obtained. Betadine was used to cleanse the area. Ethyl chloride was sprayed to numb the area. 1 cc of lidocaine plain with 0.5 cc of methylprednisolone at 40 mg per cc was given. Band-aid was applied.)
- INJECTION OF METHYLPREDNISOLONE ACETATE, NOW (96372) Routine ()
- Patient Education: Carpal Tunnel Syndrome: wrist
- How to access health information online
- Home exercise program for the wrist

PAIN OF RIGHT HAND (M79.641)Current Plans:**CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)**Current Plans:

- CARPAL TUNNEL INJECTION (20526) Routine (Verbal consent was obtained. Betadine was used to cleanse the area. Ethyl chloride was sprayed to numb the area. 1 cc of lidocaine plain with 0.5 cc of methylprednisolone at 40 mg per cc was given. Band-aid was applied.)
- INJECTION OF METHYLPREDNISOLONE ACETATE, NOW (96372) Routine ()

NUMBNESS AND TINGLING (R20.0)Current Plans:

CERVICALGIA (M54.2)Current Plans:**PAIN OF LEFT HAND (M79.642)**Current Plans:

- Cervical recommendations: Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord: Neck pain radiculopathy: Status post ACDF FUSED: Residual left arm numbness:
 1. Adjacent level protocol
 2. Bilateral trigger finger : Se Dr Donovan;
 3. MRI cervical for persistent left arm numbness: ALD vs. chronic nerve damage ..But I want him to see Dr Donovan first.

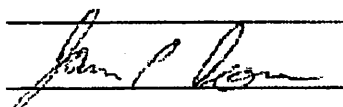
Lumbar Recommendations: Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5; lower back pain

1. Cautious use of NSAIDs.
2. Continued therapy
3. Epidural injection at L4-5 with facet blocks series option
4. If pain is not improved can consider transforaminal discectomy at L4-5(MENDS endoscopic assistance) L4 and L5 and S1 medial lateral branch neurotomy as alternative to fusion(HOLD)

Bilateral Hand Recommendations: Dupuytren's disease of the palm with cord, numbness tingling/carpal tunnel, trigger digits

1. 7/17/24-right and left carpal tunnel injections given - Repeated on 9/17/24
2. Continue bilateral wrist bracing while sleeping, discussed avoidance of aggravating activities and ergonomic changes
3. Discussed Dupuytren's disease of the palm and treatment options if symptoms persist
4. consider surgery carpal tunnel release

Follow-up: 8 weeks repeat evaluation



JAMES DONOVAN

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JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

DOS: 04/08/2025

A summary of our findings are below beginning with the patient's history with regards to their spinal diagnosis, followed by our assessment and recommendations. Following that are our exam findings at the time of the visit. If you have any questions regarding your patient's care, please do not hesitate to call us at any time.

Attending Surgeon: VICTOR M. HAYES MD

History of Present Illness (JAMES DONOVAN 04/09/2025 01:01 PM)

Patient words: 6-week postop left endoscopic carpal tunnel release and left ring finger open A1 pulley release. Patient doing well. Still having some numbness and tingling in the hand and fingers. Feels some stiffness in the ring finger. No catching or locking of the finger. Still has right hand pain with locking clicking of the long finger and sometimes tingling of the hand.

The patient is a 58 year old male who presents to discuss consultation. Follow up to discuss Orthopedic consult (The patient comes in today s/p Left Carpal Tunnel and Trigger finger release on 2/17/25. He state he has continued complaints of left hand and wrist pain with numbness/paresthesias but notes his trigger finger has improved. He also reports secondary complaints of bilateral neck pain associated with occipital headaches and mid back pain with left rib pain and lower back pain at the bilateral beltline associated with left foot numbness. His primary complaint today is his left hand pain. The patient reports his pain level today is a 2 on a 0-10 pain scale. He is currently taking Advil PRN for pain. He completed Chiropractic treatments in 2022 and currently utilizes HEP of stretching for his hand. The patient presents to the office today to discuss further treatments options.).

Allergies (Chelsea Yearwood; 04/08/2025 08:55 AM)**No Known Drug Allergies** 03/04/2021**Past Medical History** (Chelsea Yearwood; 04/08/2025 08:55 AM)**No pertinent Spine Related past medical history****MUSCLE SPASM (M62.838)****PRE-OP CHEST EXAM (Z01.811)****CORD COMPRESSION (G95.20)****ARTHRODESIS STATUS (Z98.1)****PAIN OF RIGHT HAND (M79.641)****LUMBAGO SYNDROME (M54.5)****CERVICAL DISC DISORDER (M50.90)****PAIN IN THORACIC SPINE (M54.6)****DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (M51.26)****FOLLOW-UP EXAMINATION AFTER ORTHOPEDIC SURGERY (Z09)****CARPAL TUNNEL SYNDROME OF RIGHT WRIST (G56.01)****PAIN OF LEFT HAND (M79.642)****TRIGGER RING FINGER OF LEFT HAND (M65.342)****CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)****MOTOR VEHICLE ACCIDENT (V89.2XXA)****ENCOUNTER FOR REMOVAL OF SUTURES (Z48.02)****TRIGGER MIDDLE FINGER OF RIGHT HAND (M65.331)**

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Wednesday, April 9, 2025

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CERVICALGIA (M54.2)
NUMBNESS AND TINGLING (R20.0)
DUPUYTREN CONTRACTURE (M72.0)

Family History (Chelsea Yearwood; 04/08/2025 08:55 AM)

No pertinent family history: With regards to Spinal Disorder

Family History Reviewed - no changes From Previous Visit

Social History (Chelsea Yearwood; 04/08/2025 08:55 AM)

Marital status: Divorced

Current tobacco use: Never smoker

No Drug Use

Non Drinker/No Alcohol Use

Current Household Members; Alone

Current work status: Self-employed; Internet Sales, Carparts & Mechanic

Medication History (Chelsea Yearwood; 04/08/2025 08:55 AM)

Advil (1 (one) Oral as needed) Specific strength unknown - Active.

Medications Reconciled.

Other Problems (Chelsea Yearwood; 04/08/2025 08:55 AM)

EXERCISE COUNSELING (Z71.82)

Past Surgical (Chelsea Yearwood; 04/08/2025 08:55 AM)

ACDF @ C6-7 (03/09/2022); with Dr. Victor M Hayes, M.D

Diagnostic Studies (Chelsea Yearwood; 04/08/2025 08:55 AM)

MRI, Lumbar Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 anterolisthesis of L5 over S1. Grade 1 retrolisthesis of L1 over L2 and L2 over L3. Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5.

MRI, Cervical Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 retrolisthesis of C2 over C3 and C3 over C4. Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord.

Cervical Spine X-ray (07/08/2024); In Office 2 view AP/ Lat (VMH): Instrumentation in good position. FUSED Well

03/17/2022: In Office 2 view AP/ Lat (VMH): Instrumentation in good position.

Review of Systems (Chelsea Yearwood; 04/08/2025 08:55 AM)

General Present- Fatigue and Night Sweats. Not Present- Fever and Persistent Infections.

Skin Not Present- Bruising.

HEENT Not Present- Headache.

Neck Present- Neck Pain (pain radiates into bilateral periscapular shoulders and upper back (s/p ACDF at C6-7 on 03/09/2022)) and Neck Stiffness.

Respiratory Not Present- Dyspnea.

Breast Not Present- Breast Mass.

Cardiovascular Not Present- Leg Cramps.

Gastrointestinal Not Present- Heartburn.

Musculoskeletal Present- Back Pain (upper back, midline at midback with anterior left rib pain and middle at low back), Backache and Joint Pain (bilateral hands).

Neurological Present- Numbness (bilateral arms and hands), Paresthesias (bilateral arms and hands) and Trouble walking (antalgic gait and balance issues). Not Present- Incontinence Stool, Incontinence Urine, Unsteadiness, Vertigo and Weakness In Extremities.

Psychiatric Present- Change in Sleep Pattern, Mood changes and Trouble Falling Asleep. Not Present- Anxiety.

Endocrine Not Present- Appetite Changes.

Hematology Not Present- Abnormal Bleeding.

Vitals (Chelsea Yearwood; 04/08/2025 09:00 AM)

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Wednesday, April 9, 2025

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04/08/2025 08:55 AM

Weight: 195 lb **Height:** 72 in**Body Surface Area:** 2.11 m² **Body Mass Index:** 26.45 kg/m²**Pain level:** 2/10**Pulse:** 88 (Regular) **Resp.:** 16 (Unlabored) **P. OX:** 99% (Room air)**BP:** 134/94 Manual (Sitting, Left Arm, Standard)**Physical Exam** (JAMES DONOVAN: 04/09/2025 01:01 PM)

The physical exam findings are as follows:

General**Mental Status** - Alert.**General Appearance** - Cooperative, Well groomed, Not in acute distress.**Orientation** - Oriented X3.**Build & Nutrition** - Well nourished.**Musculoskeletal****Upper Extremity****Elbow - Functional Testing** - Pronator Teres Syndrome Test negative, (R), Pronator Teres Syndrome Test negative, (L), Tinel's Sign negative, (R), Tinel's Sign negative, (L).**Hand/Wrist - Functional Testing** - Tinel's Sign negative, (R), Tinel's Sign negative, (L). **Hand - Evaluation of related systems reveals** - no digital clubbing or cyanosis. **Examination of the right hand reveals** - no tenderness to palpation, no pain, normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities. **Examination of the left hand reveals** - normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities. Note: Carpal tunnel and ring finger A1 pulley incisions clean dry and intact. Patient able to make complete fist.**Deformities/Malalignments/Discrepancies** - Dupuytren's contracture, (R) (cord formation) and Dupuytren's contracture, (L) (cord formation).**Phalanges:****Right 3rd Digit - Deformities/Malalignments/Discrepancies** - Trigger finger.**Assessment & Plan** (JAMES DONOVAN: 04/09/2025 01:03 PM)**MOTOR VEHICLE ACCIDENT (V89.2XXA)**Current Plans:**CERVICAL DISC DISORDER (M50.90)**Current Plans:**PAIN IN THORACIC SPINE (M54.6)**Current Plans:**LUMBAGO SYNDROME (M54.5)**Current Plans:

- Cervical recommendations: Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord: Neck pain radiculopathy: Status post ACDF FUSED: Residual left arm numbness:
 1. Adjacent level protocol
 2. Bilateral trigger finger: Se Dr Donovan:
 3. MRI cervical for persistent left arm numbness: ALD vs. chronic nerve damage ..But I want him to see Dr Donovan first.

Lumbar Recommendations: Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5; lower back pain

1. Cautious use of NSAIDs.
2. Continued therapy
3. Epidural injection at L4-5 with facet blocks series option
4. If pain is not improved can consider transforaminal discectomy at L4-5 (MENDS endoscopic assistance) L4 and L5 and S1 medial lateral branch neurectomy as alternative to fusion (HOLD)

Bilateral Hand Recommendations: Dupuytren's disease of the palm with cord, numbness tingling/carpal tunnel, trigger digits

1. 7/17/24-right and left carpal tunnel injections given - Repeated on 9/17/24-minimal improvement
2. Continue bilateral wrist bracing while sleeping, discussed avoidance of aggravating activities and ergonomic changes
3. Discussed Dupuytren's disease of the palm and treatment options if symptoms persist
4. consider surgery carpal tunnel release, discussed staging left versus right side
5. 2/17/25 -left endoscopic carpal tunnel release and left ring finger open A1 pulley release

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Wednesday, April 9, 2025

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6. 3/7/25 -sutures removed in office today. Discussed increasing weight lifting 1 pound more per week up to 6 pounds and 6 weeks postoperatively. Discussed it may take up to 9 months for nerve regeneration and healing. Discussed the potential risk for some aspect of permanent nerve damage. Encourage patient to work on active and passive range of motion of the fingers.
730/8/25-increasing activity no restrictions left upper extremity at this time. Discussed right hand trigger finger and numbness tingling. Also discussed cervical spine possible-phenomenon

Follow-up: 6 weeks repeat evaluation.

CERVICALGIA (M54.2)

Current Plans:

NUMBNESS AND TINGLING (R20.0)

Current Plans:

DUPUYTREN CONTRACTURE (M72.0)

Current Plans:

PAIN OF LEFT HAND (M79.642)

Current Plans:

- Home exercise program for the hand/fingers

TRIGGER RING FINGER OF LEFT HAND (M65.342)

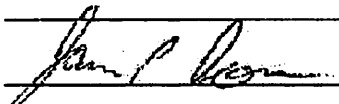
Current Plans:

- Patient Education: Trigger Finger: hand

CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)

Current Plans:

- Patient Education: Carpal Tunnel Syndrome: hand pain



JAMES DONOVAN

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JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

DOS: 05/20/2025

A summary of our findings are below beginning with the patient's history with regards to their spinal diagnosis, followed by our assessment and recommendations. Following that are our exam findings at the time of the visit. If you have any questions regarding your patient's care, please do not hesitate to call us at any time.

History of Present Illness (JAMES DONOVAN 05/21/2025 08:47 AM)

Patient words: Patient presents 3 months postop left endoscopic carpal tunnel release and left ring finger open A1 pulley release. Patient doing well. Still having some numbness and tingling in the hand and fingers. Feels some stiffness in the ring finger. No catching or locking of the finger. Also still having some pain in the palm.

The patient is a 58 year old male who presents to discuss consultation. Follow up to discuss Orthopedic consult (The patient returns s/p a left endoscopic carpal tunnel release and left ring finger open A1 pulley release on 02/17/2025 with continued complaints of residual left hand pain with wrist pain associated with numbness and paresthesias. He also reports secondary complaints of bilateral neck pain associated with occipital headaches and pain that radiate into bilateral arms to the hands associated with numbness and paresthesias and mid back pain with left rib pain and lower back pain at the bilateral beltline associated with left foot numbness. The patient is taking Advil as needed and is continuing home exercises. He states he has been increasing his activity level and lifts up to approximately 25-30 pounds. The patient reports he feels as though his hand symptoms are not improving.).

Allergies (Autumn Altenburg; 05/20/2025 09:31 AM)

No Known Drug Allergies 03/04/2021

Past Medical History (Autumn Altenburg; 05/20/2025 09:31 AM)

No pertinent Spine Related past medical history

FOLLOW-UP EXAMINATION AFTER ORTHOPEDIC SURGERY (Z09)

DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (M51.26)

CARPAL TUNNEL SYNDROME OF RIGHT WRIST (G56.01)

ENCOUNTER FOR REMOVAL OF SUTURES (Z48.02)

TRIGGER MIDDLE FINGER OF RIGHT HAND (M65.331)

MUSCLE SPASM (M62.838)

PRE-OP CHEST EXAM (Z01.811)

CORD COMPRESSION (G95.20)

ARTHRODESIS STATUS (Z98.1)

PAIN OF RIGHT HAND (M79.641)

NUMBNESS AND TINGLING (R20.0)

DUPUYTREN CONTRACTURE (M72.0)

PAIN OF LEFT HAND (M79.642)

TRIGGER RING FINGER OF LEFT HAND (M65.342)

CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)

MOTOR VEHICLE ACCIDENT (V89.2XXA)

CERVICAL DISC DISORDER (M50.90)

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Wednesday, May 21, 2025

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PAIN IN THORACIC SPINE (M54.6)
LUMBAGO SYNDROME (M54.5)
CERVICALGIA (M54.2)

Family History (Autumn Altenburg; 05/20/2025 09:31 AM)

Family History Reviewed - no changes From Previous Visit

Social History (Autumn Altenburg; 05/20/2025 09:31 AM)

Marital status: Divorced

Current tobacco use: Never smoker

No Drug Use

Non Drinker/No Alcohol Use

Current Household Members; Alone

Current work status: Self-employed; Internet Sales, Carparts & Mechanic

Medication History (Autumn Altenburg; 05/20/2025 09:42 AM)

Advil (1 (one) Oral as needed) Specific strength unknown - Active.

Medications Reconciled.

Other Problems (Autumn Altenburg; 05/20/2025 09:31 AM)

EXERCISE COUNSELING (Z71.82)

Past Surgical (Autumn Altenburg; 05/20/2025 09:32 AM)

ACDF @ C6-7 (03/09/2022); with Dr. Victor M Hayes, M.D

left endoscopic carpal tunnel release and left ring finger open A1 pulley release (02/17/2025); with Dr. Donovan

Diagnostic Studies (Autumn Altenburg; 05/20/2025 09:31 AM)

MRI, Lumbar Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 anterolisthesis of L5 over S1. Grade 1 retrolisthesis of L1 over L2 and L2 over L3. Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5.

MRI, Cervical Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 retrolisthesis of C2 over C3 and C3 over C4. Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord.

Cervical Spine X-ray (07/08/2024); In Office 2 view AP/ Lat (VMH): Instrumentation in good position. FUSED Well

03/17/2022: In Office 2 view AP/ Lat (VMH): Instrumentation in good position.

Review of Systems (Autumn Altenburg; 05/20/2025 09:42 AM)

General Present- Fatigue and Night Sweats. Not Present- Fever and Persistent Infections.

Skin Not Present- Bruising.

HEENT Not Present- Headache.

Neck Present- Neck Pain (residual neck pain that radiates into bilateral arms to hands (s/p ACDF at C6-7 on 03/09/2022)) and **Neck Stiffness.**

Respiratory Not Present- Dyspnea.

Breast Not Present- Breast Mass.

Cardiovascular Not Present- Leg Cramps.

Gastrointestinal Not Present- Heartburn.

Musculoskeletal Present- Back Pain (upper back, midline at midback with anterior left rib pain and middle at low back), **Backache** and **Joint Pain** (bilateral hands).

Neurological Present- Numbness (bilateral arms and hands), **Paresthesias** (bilateral arms and hands) and **Trouble walking** (antalgic gait and balance issues). Not Present- Incontinence Stool, Incontinence Urine, Unsteadiness, Vertigo and Weakness In Extremities.

Psychiatric Present- Change in Sleep Pattern, Mood changes and Trouble Falling Asleep. Not Present- Anxiety.

Endocrine Not Present- Appetite Changes.

Hematology Not Present- Abnormal Bleeding.

Vitals (Autumn Altenburg; 05/20/2025 09:43 AM)

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Wednesday, May 21, 2025

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05/20/2025 09:42 AM

Weight: 212 lb **Height:** 72 in**Body Surface Area:** 2.18 m² **Body Mass Index:** 28.75 kg/m²**Pain level:** 3/10**Pulse:** 74 (Regular) **Resp.:** 16 (Unlabored) **P. OX:** 98% (Room air)**BP:** 136/87 Manual (Sitting, Left Arm, Standard)**Physical Exam** (JAMES DONOVAN: 05/21/2025 08:48 AM)

The physical exam findings are as follows:

General**Mental Status** - Alert.**General Appearance** - Cooperative, Well groomed, Not in acute distress.**Orientation** - Oriented X3.**Build & Nutrition** - Well nourished.**Musculoskeletal****Upper Extremity****Elbow - Functional Testing** - Pronator Teres Syndrome Test negative, (R), Pronator Teres Syndrome Test negative, (L), Tinel's Sign negative, (R), Tinel's Sign negative, (L).**Hand/Wrist: Wrist - Functional Testing** - Tinel's Sign negative, (R), Tinel's Sign negative, (L). **Hand - Evaluation of related systems reveals** - no digital clubbing or cyanosis. **Examination of the right hand reveals** - no tenderness to palpation, no pain, normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities. **Examination of the left hand reveals** - normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities. Note: Carpal tunnel and ring finger A1 pulley incisions clean dry and intact. Patient able to make complete fist.**Deformities/Malalignments/Discrepancies** - Dupuytren's contracture, (R) (cord formation) and Dupuytren's contracture, (L) (cord formation).**Phalanges:****Right 3rd Digit - Deformities/Malalignments/Discrepancies** - Trigger finger.**Assessment & Plan** (JAMES DONOVAN: 05/21/2025 08:48 AM)**CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)****Current Plans:**

- HAND THERAPY INITIAL EXAMINATION AND PLANNING FOR REHABILITATION PROGRAM Routine ()
- **Referral to Hand Therapy**
Evaluate and Treat: 3-month status post left endoscopic carpal tunnel release and open left long finger A1 pulley release. Range of motion and grip strength exercises. Desensitization exercises. TENS and ultrasound as needed. Warm and cold compresses as needed. Home therapy exercises.
3 times per week
4 weeks

TRIGGER RING FINGER OF LEFT HAND (M65.342)**Current Plans:**

- Patient Education: Trigger Finger: hand
- How to access health information online
- Home exercise program for the hand/fingers

PAIN OF LEFT HAND (M79.642)**Current Plans:****DUPUYTREN CONTRACTURE (M72.0)****Current Plans:****LUMBAGO SYNDROME (M54.5)**

Current Plans:

- Cervical recommendations: Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord: Neck pain radiculopathy: Status post ACDF FUSED: Residual left arm numbness:
 1. Adjacent level protocol
 2. Bilateral trigger finger : Se Dr Donovan;
 3. MRI cervical for persistent left arm numbness: ALD vs. chronic nerve damage ..But I want him to see Dr Donovan first.

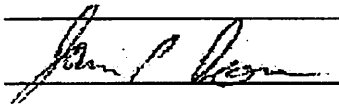
Lumbar Recommendations: Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5; lower back pain

1. Cautious use of NSAIDs.
2. Continued therapy
3. Epidural injection at L4-5 with facet blocks series option
4. If pain is not improved can consider transforaminal discectomy at L4-5(MENDS endoscopic assistance) L4 and L5 and S1 medial lateral branch neurotomy as alternative to fusion(HOLD)

Bilateral Hand Recommendations: Dupuytren's disease of the palm with cord, numbness tingling/carpal tunnel, trigger digits

1. 7/17/24-right and left carpal tunnel injections given - Repeated on 9/17/24-minimal improvement
2. Continue bilateral wrist bracing while sleeping, discussed avoidance of aggravating activities and ergonomic changes
3. Discussed Dupuytren's disease of the palm and treatment options if symptoms persist
4. consider surgery carpal tunnel release, discussed staging left versus right side
5. 2/17/25 -left endoscopic carpal tunnel release and left ring finger open A1 pulley release
6. 3/7/25 -sutures removed in office today. Discussed increasing weight lifting 1 pound more per week up to 6 pounds and 6 weeks postoperatively. Discussed it may take up to 9 months for nerve regeneration and healing. Discussed the potential risk for some aspect of permanent nerve damage. Encourage patient to work on active and passive range of motion of the fingers.
7. 3/8/25-increasing activity no restrictions left upper extremity at this time. Discussed right hand trigger finger and numbness tingling. Also discussed cervical spine possible-phenomenon
8. 5/20/25 -begin outpatient hand therapy to work on grip strength and range of motion exercises. Discussed that it may take up to 9 months for nerve regeneration and healing.

Follow-up: 6 weeks repeat evaluation.



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JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

DOS: 07/29/2025

A summary of our findings are below beginning with the patient's history with regards to their spinal diagnosis, followed by our assessment and recommendations. Following that are our exam findings at the time of the visit. If you have any questions regarding your patient's care, please do not hesitate to call us at any time.

History of Present Illness (JAMES DONOVAN 07/29/2025 02:47 PM)

Patient words: (2/17/25) postop left endoscopic carpal tunnel release and left ring finger open A1 pulley release. Patient doing well. Still having some numbness and tingling in the hand and fingers. Feels some stiffness in the ring finger. No catching or locking of the finger. Also still having some pain in the palm. Still some pain of right hand. Has been in hand therapy.

The patient is a 58 year old male who presents to discuss consultation. Follow up to discuss Orthopedic consult (The patient returns today for a 6 week follow up on his left hand associated with numbness/paresthesias and shooting pains and twitching in the ring finger and thumb and right hand pain. He states he continues the HEP given to him by his Physical therapist daily but has not noticed much improvement. He also reports secondary complaints of bilateral neck pain associated with occipital headaches and pain that radiate into bilateral arms to the hands associated with numbness and paresthesias and mid back pain with left rib pain and lower back pain at the bilateral beltline associated with left foot numbness. The patient is taking Advil as needed for pain. The patient is here to further discuss his symptoms.)

Allergies (Destiny Ortega; 07/29/2025 10:22 AM)

No Known Drug Allergies 03/04/2021

Past Medical History (Destiny Ortega; 07/29/2025 10:22 AM)

No pertinent Spine Related past medical history

PRE-OP CHEST EXAM (Z01.811)

MUSCLE SPASM (M62.838)

CORD COMPRESSION (G95.20)

ARTHRODESIS STATUS (Z98.1)

PAIN OF RIGHT HAND (M79.641)

FOLLOW-UP EXAMINATION AFTER ORTHOPEDIC SURGERY (Z09)

DISPLACEMENT OF LUMBAR INTERVERTEBRAL DISC WITHOUT MYELOPATHY (M51.26)

CARPAL TUNNEL SYNDROME OF RIGHT WRIST (G56.01)

ENCOUNTER FOR REMOVAL OF SUTURES (Z48.02)

TRIGGER MIDDLE FINGER OF RIGHT HAND (M65.331)

NUMBNESS AND TINGLING (R20.0)

CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)

TRIGGER RING FINGER OF LEFT HAND (M65.342)

PAIN OF LEFT HAND (M79.642)

DUPUYTREN CONTRACTURE (M72.0)

LUMBAGO SYNDROME (M54.5)

MOTOR VEHICLE ACCIDENT (V89.2XXA)

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Tuesday, July 29, 2025

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CERVICAL DISC DISORDER (M50.90)
PAIN IN THORACIC SPINE (M54.6)
CERVICALGIA (M54.2)

Family History (Destiny Ortega; 07/29/2025 10:22 AM)

Family History Reviewed - no changes From Previous Visit
No pertinent family history: With regards to Spinal Disorder

Social History (Destiny Ortega; 07/29/2025 10:22 AM)

Marital status: Divorced

Current tobacco use: Never smoker

No Drug Use

Non Drinker/No Alcohol Use

Current Household Members; Alone

Current work status: Self-employed; Internet Sales, Carparts & Mechanic

Medication History (Destiny Ortega; 07/29/2025 10:22 AM)

Advil (1 (one) Oral as needed) Specific strength unknown - Active.

Medications Reconciled.

Other Problems (Destiny Ortega; 07/29/2025 10:22 AM)

EXERCISE COUNSELING (Z71.82)

Past Surgical (Destiny Ortega; 07/29/2025 10:22 AM)

ACDF @ C6-7 (03/09/2022); with Dr. Victor M Hayes, M.D

left endoscopic carpal tunnel release and left ring finger open A1 pulley release (02/17/2025); with Dr. Donovan

Diagnostic Studies (Destiny Ortega; 07/29/2025 10:22 AM)

MRI, Lumbar Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 anterolisthesis of L5 over S1. Grade 1 retrolisthesis of L1 over L2 and L2 over L3. Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5.

MRI, Cervical Spine (02/10/2021); ADG: PACS and reports reviewed: Grade 1 retrolisthesis of C2 over C3 and C3 over C4. Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord.

Cervical Spine X-ray (07/08/2024); In Office 2 view AP/ Lat (VMH): Instrumentation in good position. FUSED Well

03/17/2022: In Office 2 view AP/ Lat (VMH): Instrumentation in good position.

Review of Systems (Destiny Ortega; 07/29/2025 10:21 AM)

General Present- Fatigue and Night Sweats. Not Present- Fever and Persistent Infections.

Skin Not Present- Bruising.

HEENT Not Present- Headache.

Neck Present- Neck Pain (residual neck pain that radiates into bilateral arms to hands (s/p ACDF at C6-7 on 03/09/2022)) and **Neck Stiffness.**

Respiratory Not Present- Dyspnea.

Breast Not Present- Breast Mass.

Cardiovascular Not Present- Leg Cramps.

Gastrointestinal Not Present- Heartburn.

Musculoskeletal Present- Back Pain (upper back, midline at midback with anterior left rib pain and middle at low back), **Backache**

and **Joint Pain** (bilateral hands).

Neurological Present- Numbness (bilateral arms and hands), **Paresthesias** (bilateral arms and hands) and **Trouble walking** (antalgic gait and balance issues). Not Present- Incontinence Stool, Incontinence Urine, Unsteadiness, Vertigo and Weakness In Extremities.

Psychiatric Present- Change in Sleep Pattern, Mood changes and Trouble Falling Asleep. Not Present- Anxiety.

Endocrine Not Present- Appetite Changes.

Hematology Not Present- Abnormal Bleeding.

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Tuesday, July 29, 2025

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000236

Vitals (Destiny Ortega; 07/29/2025 10:29 AM)

07/29/2025 10:24 AM

Weight: 212 lb **Height:** 72 in**Body Surface Area:** 2.18 m² **Body Mass Index:** 28.75 kg/m²**Pulse:** 95 (Regular) **Resp.:** 17 (Unlabored) **P. OX:** 98% (Room air)**BP:** 140/108 Manual (Sitting, Left Arm, Standard)**Physical Exam** (JAMES DONOVAN; 07/29/2025 02:48 PM)

The physical exam findings are as follows:

General**Mental Status** - Alert.**General Appearance** - Cooperative, Well groomed, Not in acute distress.**Orientation** - Oriented X3.**Build & Nutrition** - Well nourished.**Musculoskeletal****Upper Extremity****Elbow - Functional Testing** - Pronator Teres Syndrome Test negative, (R), Pronator Teres Syndrome Test negative, (L), Tinel's Sign negative, (R), Tinel's Sign negative, (L).**Hand/Wrist: Wrist - Functional Testing** - Tinel's Sign negative, (R), Tinel's Sign negative, (L). **Hand - Evaluation of related systems reveals** - no digital clubbing or cyanosis. **Examination of the right hand reveals** - no tenderness to palpation, no pain, normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities. **Examination of the left hand reveals** - normal range of motion, no crepitus, no instability or laxity and no known fractures or deformities. Note: Carpal tunnel and ring finger A1 pulley incisions clean dry and intact. Patient able to make complete fist.**Strength and Tone: Right - Grip Strength** - 120 LBS. **Pulp to Pulp Pinch Strength** - 16 LBS. **Left - Grip Strength** - 65 LBS. **Pulp to Pulp Pinch Strength** - 13 LBS. **Hand - Deformities/Malalignments/Discrepancies** - Dupuytren's contracture, (R) (cord formation) and Dupuytren's contracture, (L) (cord formation).**Phalanges:****Right: 3rd Digit - Deformities/Malalignments/Discrepancies** - Trigger finger.**Assessment & Plan** (JAMES DONOVAN; 07/29/2025 02:49 PM)**MOTOR VEHICLE ACCIDENT (V89.2XXA)**Current Plans:**CERVICALGIA (M54.2)**Current Plans:**LUMBAGO SYNDROME (M54.5)**Current Plans:

- Cervical recommendations: Broad-based posterocentral disc herniation at C3-4. Large broad-based posterocentral disc herniation at C6-7 with moderate bilateral neural foraminal narrowing and mild central canal stenosis with compression on the spinal cord: Neck pain radiculopathy: Status post ACDF FUSED: Residual left arm numbness:
 1. Adjacent level protocol
 2. Bilateral trigger finger : Se Dr Donovan:
 3. MRI cervical for persistent left arm numbness: ALD vs. chronic nerve damage ..But I want him to see Dr Donovan first.

Lumbar Recommendations: Disc bulge at L1-2 with left-sided neural foraminal narrowing and a mild facet joint effusion. Disc bulge at L2-3 with a mild facet joint effusion. Disc bulge at L3-4 with bilateral neural foraminal narrowing. Broad-based posterocentral disc herniation at L4-5; lower back pain

1. Cautious use of NSAIDs.
2. Continued therapy
3. Epidural injection at L4-5 with facet blocks series option
4. If pain is not improved can consider transforaminal discectomy at L4-5 (MENDS endoscopic assistance) L4 and L5 and S1 medial lateral branch neurotomy as alternative to fusion (HOLD)

Bilateral Hand Recommendations: Dupuytren's disease of the palm with cord, numbness tingling/carpal tunnel, trigger digits

1. 7/17/24-right and left carpal tunnel injections given - Repeated on 9/17/24-minimal improvement
2. Continue bilateral wrist bracing while sleeping, discussed avoidance of aggravating activities and ergonomic changes
3. Discussed Dupuytren's disease of the palm and treatment options if symptoms persist
4. consider surgery carpal tunnel release, discussed staging left versus right side
5. 2/17/25 -left endoscopic carpal tunnel release and left ring finger open A1 pulley release
6. 3/7/25 -sutures removed in office today. Discussed increasing weight lifting 1 pound more per week up to 6 pounds and 6 weeks postoperatively. Discussed it may take up to 9 months for nerve regeneration and healing. Discussed the potential risk for some aspect of permanent nerve damage. Encourage patient to work on active and passive range of motion of the fingers.

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Tuesday, July 29, 2025

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7. 3/8/25-increasing activity no restrictions left upper extremity at this time. Discussed right hand trigger finger and numbness tingling. Also discussed cervical spine possible-phenomenon
8. 5/20/25 -begin outpatient hand therapy to work on grip strength and range of motion exercises. Discussed that it may take up to 9 months for nerve regeneration and healing.
9. 7/29/2025-improvement for trigger finger. Still having the tingling of the hand and some shaking of the hands. Discussed neurology referral

Follow-up: As needed or flareup

PAIN OF RIGHT HAND (M79.641)Current Plans:**NUMBNESS AND TINGLING (R20.0)**Current Plans:

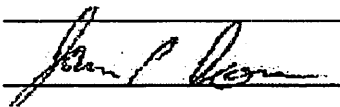
- COMPLETE ELECTROMYOGRAPHY (EMG) WITH NERVE CONDUCTION STUDIES OF EACH EXTREMITY (95886) Routine ()
- Referred to Neurology, (Neurology)

PAIN OF LEFT HAND (M79.642)Current Plans:**TRIGGER RING FINGER OF LEFT HAND (M65.342)**Current Plans:**CARPAL TUNNEL SYNDROME OF LEFT WRIST (G56.02)**Current Plans:

- Patient Education: Carpal Tunnel Syndrome: hand pain
- Home exercise program for the hand/fingers
- How to access health information online

EXERCISE COUNSELING (Z71.82)Current Plans:

- Education and Counseling
- Education and Counseling



JAMES DONOVAN

Elite Spine and Orthopedics



2040 SHORT AVE, SUITE 100
ODESSA, FL 33556
Phone: (727) 372-9922
Fax: (727) 372-8477

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

DOS: 07/08/2024

A summary of our findings are below beginning with the patient's history with regards to their spinal diagnosis, followed by our assessment and recommendations. Following that are our exam findings at the time of the visit. If you have any questions regarding your patient's care, please do not hesitate to call us at any time.

Attending Surgeon: VICTOR M. HAYES MD

History of Present Illness**Assessment & Plan** (VICTOR M. HAYES MD; 07/08/2024 09:30 AM)**CERVICALGIA (M54.2)**Current Plans:

- X-RAY EXAM OF NECK SPINE (72040) Routine ()

VICTOR M. HAYES MD

JEFFREY PLUMMER

Patient #: 37726

DOB: [REDACTED]

Monday, July 8, 2024

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DOCTOR/HOSPITAL LIEN

PATIENT IS RESPONSIBLE FOR ALL EXPENSES INCURRED REGARDLESS OF THE OUTCOME OF ANY SETTLEMENT, LITIGATION OR TRIAL

PATIENT: Jeff L. Plummer
HEALTH PROVIDER: Paradise Palms Chiropractic
19011 Dale Mabry Hwy.
Lutz, FL 33548
DATE OF ACCIDENT: January 05, 2021

PROTECTION OF OUTSTANDING CHARGES: If the above-named client recovers money damages from any person or entity responsible for charges incurred by the above-named health provider, we agree to withhold from any check or draft in which we are an additional named payee, sufficient funds, after deduction of attorney's fees and costs, to pay any outstanding medical bills in our possession and costs for any and all undisputed charges owed to you in connection with the accident or event giving rise to and covered by the recovery and not covered by any collateral source.

AMOUNT PROTECTED: The amount protected will only be the accident-related medical charges which are above and beyond those for which the insurance carrier(s) is/are legally responsible. For this letter to be valid, you will need to submit your bill to any applicable automobile and/or health insurance coverage which our client may have (e.g. PIP, group coverage, Medicare, etc.). It is the health provider's obligation to furnish us with periodic updates of outstanding charges. Otherwise, we will rely on previously received records in seeking reimbursement from the tortfeasor. Under no circumstances will we withhold a sum larger than that submitted to the tortfeasor for reimbursement.

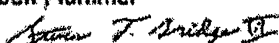
BALANCE CONFIRMATION: We will use best efforts to request a balance confirmation when recovery is imminent. If we fail to receive a written response within five (5) days of mailing, we will presume that the balance has been paid in full.

PRO RATA DISTRIBUTION IF INADEQUATE RECOVERY: If the net recovery is less than the total outstanding charges owed to all health providers covered by a Letter of Protection or any other lien holder, such funds will be distributed on a pro rata basis with the patient still responsible for the difference.

OUR RESPONSIBILITY ON FORENSIC SERVICES: This law firm acknowledges independent responsibility to the health providers for charges incurred for medical records and witness fees.

DISPUTES: If our client disputes any of your outstanding charges or claims a setoff and we are unable to resolve the issue, we will deposit the amount of the disputed charge/setoff into the Court Registry for judicial determination.

APPROVAL REQUIRED: This agreement becomes effective when all parties (client/patient, health provider and attorney) approve it in writing in the space provided below and return it to our office.


Jeff Plummer




Health Provider